

UHC Survey Year 2 — Official Codebook

Generated 2026-08-20 05:10 UTC from the build artifacts (dictionary + question text + skip-logic/validation spec) — regenerate, never hand-edit. Variable documentation follows the DDI-Codebook 2.5 element set used by the PSA Data Archive (PSADA/NADA); presentation follows the DHS recode-manual table style.

Values in the datasets are RAW stored codes; this codebook is the labeling truth.

Missing-value conventions (Shared Codebook v0.8 / tracker #743):

categorical items: 8 / 98 = Don't know · 9 / 99 = Refused (per value set)

amount/continuous items: -98 = Don't know · -99 = Refused (negative sentinels)

harmonized Stata output maps these to .c (DK), .b (Refused), .a (skip-logic NOTAPPL)

questionnaire_number is a 12-digit string key (RR-PP-MMM-FF-CCC); breakout exports may drop a leading region zero — restore by zero-filling to 12.

Skip logic: 'Universe / applicability' = who is asked (derived from the Skip-Logic spec); outbound routing shown on the variable that triggers it. Executable truth stays in the instrument logic (.apc).

Validation & editing: per-variable rules in 'Validation rules' + the Validations sheet (HARD = block save, SOFT = warn-and-confirm, GATE = display-only) — maps to DDI's data-editing/cleanOps element at dissemination.

DDI mapping: Variable→var/@name · Label→var/lab1 · Question→var/qstn · Universe→var/universe · Values→var/catgry · Special codes→missing · packaged as DDI 2.5 at dissemination.

F1 — Facility Head Survey — v3.1.3 (2026-08-20)

Variable	Label	Question (EN, literal)	Universe / applicability	Skip / routing (outbound)	Record / Roster	Type	Values & categories
questionnaire_number	Questionnaire Number (12-digit: RR-PP-MMM-FF-CCC)	Build: F1 v3.1.3 (2026-08-20) Universal Health Care (UHC) Survey 2026- Year 2 PSA SSRCS Clearance No. DOH-2651-01 · issued July 2026 · valid until 31 July 2027 SJREB: ICF ver. 07/25/2026 · Translated Questionnaire ver. 06/05/2026			Case identifiers	numeric 12	
survey_team_leader_s_name	Survey Team Leader's Name	Survey Team Leader's Name			Field Control	alpha 50	
enumerator_s_name	Enumerator's Name	Enumerator's Name			Field Control	alpha 50	
field_validated_by	Field Validated by	Field Validated by			Field Control	alpha 50	
field_edited_by	Field Edited by	Field Edited by			Field Control	alpha 50	
date_first_visited_the_facility	Date First Visited the Facility (MMDDYYYY)	Date First Visited the Facility (MMDDYYYY)			Field Control	numeric 8	
date_of_final_visit_to_the_facility	Date of Final Visit to the Facility (MMDDYYYY)	Date of Final Visit to the Facility (MMDDYYYY)			Field Control	numeric 8	
total_number_of_visits	Total Number of Visits	Total Number of Visits			Field Control	numeric 3	
enum_result_first_visit	Result of First Visit	Result of First Visit			Field Control	numeric 1	1 = Completed 2 = Postponed 4 = Incomplete
enum_result_final_visit	Result of Final Visit	Result of Final Visit			Field Control	numeric 1	1 = Completed 2 = Postponed 4 = Incomplete 5 = Replaced
breakoff	Interview status (leave as Continue unless ending the interview early)	Interview status (leave as Continue unless ending the interview early)			Field Control	numeric 1	1 = Continue interview 2 = Respondent with 3 = Postponed / reschedule 4 = Stop — other (incomplete) 6 = Not interviewed found 7 = Not interviewed ineligible
case_disposition	Case disposition (auto)	Case disposition (auto)			Field Control	numeric 1	0 = In progress 1 = Completed 2 = Partial / not com
language_used	Language used for the interview	Language used for the interview			Field Control	alpha 20	

region_code	Region Code (PSGC)	Region Code (PSGC)			Field Control	numeric 2	
province_huc_code	Province / HUC Code (PSGC)	Province / HUC Code (PSGC)			Field Control	numeric 2	
city_municipality_code	City / Municipality Code (PSGC)	City / Municipality Code (PSGC)			Field Control	numeric 3	
facility_no	Facility Number (within municipality)	Facility Number (within municipality)			Field Control	numeric 2	
case_seq	Case Sequence (per-facility, per-instrument)	Case Sequence (per-facility, per-instrument)			Field Control	numeric 3	
region_name	Region (from PSGC)	Region (from PSGC)			Field Control	alpha 80	
province_name	Province / HUC (from PSGC)	Province / HUC (from PSGC)			Field Control	alpha 80	
city_name	City / Municipality (from PSGC)	City / Municipality (from PSGC)			Field Control	alpha 80	
classification	Classification	Classification			Health Facility and Geographic Identification	numeric 1	1 = UHC IS 2 = Non-UHC IS
region	Region	Region			Health Facility and Geographic Identification	numeric 10	0000000000 = (set a runtime)
province_huc	Province / HUC	Province / HUC			Health Facility and Geographic Identification	numeric 10	0000000000 = (set a runtime)
city_municipality	City / Municipality	City / Municipality			Health Facility and Geographic Identification	numeric 10	0000000000 = (set a runtime)
barangay	Barangay	Barangay			Health Facility and Geographic Identification	numeric 10	0000000000 = (set a runtime)
facility_name	Facility Name	Facility Name			Health Facility and Geographic Identification	alpha 100	
facility_address	Facility Address	Facility Address			Health Facility and Geographic Identification	alpha 200	
facility_gps_latitude	GPS Latitude	GPS Latitude			Facility GPS and Verification Photo	alpha 12	
facility_gps_longitude	GPS Longitude	GPS Longitude			Facility GPS and Verification Photo	alpha 12	
facility_gps_altitude	GPS Altitude (m)	GPS Altitude (m)			Facility GPS and Verification Photo	alpha 10	
facility_gps_accuracy	GPS Accuracy (m)	GPS Accuracy (m)			Facility GPS and Verification Photo	numeric 3	
facility_gps_satellites	GPS Satellites	GPS Satellites			Facility GPS and Verification Photo	numeric 2	
facility_gps_readtime	GPS Read Time (UTC)	GPS Read Time (UTC)			Facility GPS and Verification Photo	alpha 19	
verification_photo_image	Verification Photo (binary; syncs to CSWeb)				Facility GPS and Verification Photo	image	
verification_photo_filename	Verification Photo Filename	Verification Photo Filename			Facility GPS and Verification Photo	alpha 120	
capture_verification_photo	Take Verification Photo	Take Verification Photo			Facility GPS and Verification Photo	numeric 1	1 = Take verification

icf_part1	Informed Consent — read-aloud, screen 1 of 2 (study, coverage, the ask)	We work for Asian Social Project Services, Inc. (ASPSI). We are here to invite you to participate in a study about the Universal Health Care (UHC) and packages of programs like Yaman ng Kalusugan Program (YAKAP), No Balance Billing (NBB), Zero Balance Billing (ZBB), Bagong Urgent Care and Ambulatory Services (BUCAS), and Guaranteed and Accessible Medications for Outpatient Treatment (GAMOT). The Department of Health funded this study. Please let me tell you more about the study. This study aims to generate evidence on the overall experience of the general public to support continuous monitoring, evaluation, and learning of the implementation of the UHC Act and its Implementing Rules and Regulations (IRR). The questions for this survey cover facility/facility head profile, UHC implementation changes since 2019, specialized programs (YAKAP/Konsulta, NBB/ZBB, BUCAS, GAMOT), DOH licensing, PhilHealth accreditation, NBB/ZBB policies, referral systems, human resource challenges, and secondary data such as hospital census and staffing statistics. Would you like to participate as a respondent in the study? The interview may last for more or less than an hour. Universal Health Care (UHC) Survey 2026- Year 2 PSA SSRCS Clearance No. DOH-2651-01 · issued July 2026 · valid until 31 July 2027 SJREB: ICF ver. 07/25/2026 · Translated Questionnaire ver. 06/05/2026			Introduction and Informed Consent	numeric 1	1 = Continue
icf_part2	Informed Consent — read-aloud, screen 2 of 2 (privacy, risks, voluntariness)	We are committed to protecting your privacy. If you choose to participate, we will never include your name in information shared with the government or in any reports. Your name will be kept separately from your answers in a private, secure location. For this interview, it is also important to respect other people's privacy and not tell anyone else what we talked about today. With all research, there's a small chance that someone else might get to see your data. We try our best to prevent that, but if it happens, we'll tell you as soon as possible. Aside from this, there are no other risks to you if you take part in this study. As a benefit of the research, the knowledge gained may help the government and DOH better support your healthcare needs. You are free to decline participation or to stop at any time. Choosing not to participate will not result in any penalty, and you will not have to pay anything to take part in this study. Do you have any questions about the study or about what I have told you? If you have concerns or questions about your rights as a participant, you can contact: Single Joint Research Ethics Board (SJREB) Department of Health Email: sjreb@doh.gov.ph Contact No.: (02) 8651-7800 local 1326, 1328 Department of Health Name: Lindsley Jeremiah D. Villarante Email: ldvillarante@doh.gov.ph Tel: +63 (02) 8651-7800 local 1432 Asian Social Project Services, Inc. Name: Pauly Jean A. Claro Email: inquiry.aspsi.doh.uhc.survey2@gmail.com Tel: +63 917 819 6884 Universal Health Care (UHC) Survey 2026- Year 2 PSA SSRCS Clearance No. DOH-2651-01 · issued July 2026 · valid until 31 July 2027 SJREB: ICF ver. 07/25/2026 · Translated Questionnaire ver. 06/05/2026			Introduction and Informed Consent	numeric 1	1 = Continue
resp_name	Respondent name and signature	Respondent name and signature			A. Facility Head Profile	alpha 80	
resp_position	Respondent position / office	Respondent position / office			A. Facility Head Profile	alpha 80	

resp_email	Respondent email address	Respondent email address			A. Facility Head Profile	alpha 60	
resp_mobile	Respondent mobile number	Respondent mobile number			A. Facility Head Profile	alpha 20	
q1_name	1. What is your name? (Last Name, First Name, Middle Initial, Ext)	We would like to ask some of your personal information relevant to the survey. 1. What is your name? (Last Name, First Name, Middle Initial, Ext)			A. Facility Head Profile	alpha 80	
q2_facility_role	2. What is your official designation at this health facility?	2. What is your official designation at this health facility?			A. Facility Head Profile	numeric 2	1 = Rural Health Unit 2 = Physician 3 = Chief of Hospital 4 = Medical Director 5 = Hospital Administrator 6 = Nurse 7 = Municipal / City Officer 8 = Medical Officer 9 = Administrative Officer 10 = Midwife 11 = Health Promoter 12 = Rural Health Physician 99 = Other (specify)
q2_other_txt	2. What is your official designation at this health facility? — Other (specify) text	2. What is your official designation at this health facility? — Other (specify) text			A. Facility Head Profile	alpha 120	
q3_age	3. How old are you (in years), as of your last birthday?	3. How old are you (in years), as of your last birthday?			A. Facility Head Profile	numeric 2	
q4_sex	4. What is your sex assigned at birth?	4. What is your sex assigned at birth?			A. Facility Head Profile	numeric 1	1 = Male 2 = Female
q5_years_at_facility	5. In your current position, how many months/years have you worked at this health facility? Number of Years	5. In your current position, how many months/years have you worked at this health facility? Number of Years			A. Facility Head Profile	numeric 2	
q5_months_at_facility	5. In your current position, how many months/years have you worked at this health facility? Number of Months	5. In your current position, how many months/years have you worked at this health facility? Number of Months			A. Facility Head Profile	numeric 2	
q6_years_health	6. How many years have you worked in health-related position? Number of Years	6. How many years have you worked in health-related position? Number of Years			A. Facility Head Profile	numeric 2	
q6_months_health	6. How many years have you worked in health-related position? Number of Months	6. How many years have you worked in health-related position? Number of Months			A. Facility Head Profile	numeric 2	
q7_ownership	7. What type of ownership is this hospital?	Next, we will be asking questions about this health facility. 7. What type of ownership is this hospital?			B. Facility Profile	numeric 1	1 = Public 2 = Private
q8_service_level	8. What is the facility's service capacity level?	8. What is the facility's service capacity level?			B. Facility Profile	numeric 1	1 = Primary Care Facility 2 = Level 1 Hospital 3 = Level 2 Hospital 4 = Level 3 Hospital
q9_uhc_heard	9. Have you heard about Universal Health Care (UHC) prior to this survey?	We will now ask about your awareness of UHC, the changes that have been implemented in this facility since the UHC Act was passed in 2019 and if these changes were a result of the UHC Act. 9. Have you heard about Universal Health Care (UHC) prior to this survey?			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q10_has_primary_pkg	10. Does the facility have primary care packages?	10. Does the facility have primary care packages?		= No (2) → Q11	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q10_1_uhc_attrb	10.1. If yes, was it a result of the UHC Act enacted in 2019?	10.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequent

							enhanced or expanded due to UHC Act 3 = Newly implemented improved independent UHC Act 4 = Not yet implemented but planned within the next 1-2 years 5 = Other (specify)
q10_1_uhc_attrb_other_txt	10.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	10.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q11_pcb_licensing	11. Has the facility applied for a DOH primary care license?	11. Has the facility applied for a DOH primary care license?		= No (2) → Q12	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q11_1_uhc_attrb	11.1. If yes, was it a result of the UHC Act enacted in 2019?	11.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequent enhanced or expanded due to UHC Act 3 = Newly implemented improved independent UHC Act 4 = Not yet implemented but planned within the next 1-2 years 5 = Other (specify)
q11_1_uhc_attrb_other_txt	11.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	11.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q12_public_health_unit	12. Does the facility have a public health unit?	12. Does the facility have a public health unit?		in 2, 9 (No or Not applicable) → Q13	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q12_1_uhc_attrb	12.1. If yes, was it a result of the UHC Act enacted in 2019?	12.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequent enhanced or expanded due to UHC Act 3 = Newly implemented improved independent UHC Act 4 = Not yet implemented but planned within the next 1-2 years 5 = Other (specify)
q12_1_uhc_attrb_other_txt	12.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	12.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q12_2_phu_role	12.2. What is the main role of the public health unit?	12.2. What is the main role of the public health unit? READ OPTIONS OUT LOUD. SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Health promotion education 2 = Disease surveillance report 3 = Referral and patient navigation 4 = Alignment with national public health programs 5 = Other (specify)
q12_2_phu_role_other_txt	12.2. What is the main role of the public health unit? — Other (specify) text	12.2. What is the main role of the public health unit? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q13_health_promo_unit	13. Does the facility have a health promotion unit?	13. Does the facility have a health promotion unit?		in 2, 9 (No or Not applicable) → Q14	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q13_1_uhc_attrb	13.1. If yes, was it a result of the UHC Act enacted in 2019?	13.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequent enhanced or expanded due to UHC Act 3 = Newly implemented improved independent UHC Act

							4 = Not yet implemented but planned within the next 1-2 years 5 = Other (specify)
q13_1_uhc_attrb_other_txt	13.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	13.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q13_2_hpu_role	13.2. What is the main role of the health promotion unit?	13.2. What is the main role of the health promotion unit? READ OPTIONS OUT LOUD. SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Leading health education and awareness campaigns (e.g., raising awareness about public health initiatives or disseminating information about health) 2 = Conducting and coordinating health screening and promotion activities (e.g., collaborating and implementing with community units and program coordinators to promote healthy lifestyles and preventive care) 3 = Advocacy and policy formation (e.g., research campaigns, collaboration with policymakers) 4 = Resource mobilization and fundraising (e.g., securing funding, grants) 5 = Other (specify)
q13_2_hpu_role_other_txt	13.2. What is the main role of the health promotion unit? — Other (specify) text	13.2. What is the main role of the health promotion unit? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q14_new_roles	14. Has there been establishment of new roles in the facility?	14. Has there been establishment of new roles in the facility?		= No (2) → Q15	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q14_1_uhc_attrb	14.1. If yes, was it a result of the UHC Act enacted in 2019?	14.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequently enhanced or expanded due to UHC Act 3 = Newly implemented improved independent UHC Act 4 = Not yet implemented but planned within the next 1-2 years 5 = Other (specify)
q14_1_uhc_attrb_other_txt	14.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	14.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q14_2_new_roles_list	14.2. What is/are the new role/s established in this facility?	14.2. What is/are the new role/s established in this facility?			C. Universal Health Care (UHC) Implementation	alpha 240	
q15_new_depts	15. Has there been establishment of new departments in the facility?	15. Has there been establishment of new departments in the facility?		= No (2) → Q16	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q15_1_uhc_attrb	15.1. If yes, was it a result of the UHC Act enacted in 2019?	15.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequently enhanced or expanded due to UHC Act 3 = Newly implemented improved independent UHC Act 4 = Not yet implemented but planned within the next 1-2 years 5 = Other (specify)
q15_1_uhc_attrb_other_txt	15.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	15.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	

q15_2_new_depts_list	15.2. What is/are the new department/s established in this facility?	15.2. What is/are the new department/s established in this facility?			C. Universal Health Care (UHC) Implementation	alpha 240	
q16_new_buildings	16. Has there been construction of new buildings in this facility?	16. Has there been construction of new buildings in this facility?		= No (2) → Q17	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q16_1_uhc_attrib	16.1. If yes, was it a result of the UHC Act enacted in 2019?	16.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequent enhanced or expanded due to UHC Act 3 = Newly implemented improved independent of UHC Act 4 = Not yet implemented but planned within the next 1-2 years 5 = Other (specify)
q16_1_uhc_attrib_other_txt	16.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	16.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q16_2_new_buildings_purpose	16.2. What is/are the building/s being used for?	16.2. What is/are the building/s being used for?			C. Universal Health Care (UHC) Implementation	alpha 240	
q17_new_rooms	17. Has there been construction of new rooms in this facility?	17. Has there been construction of new rooms in this facility?		= No (2) → Q18	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q17_1_uhc_attrib	17.1. If yes, was it a result of the UHC Act enacted in 2019?	17.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequent enhanced or expanded due to UHC Act 3 = Newly implemented improved independent of UHC Act 4 = Not yet implemented but planned within the next 1-2 years 5 = Other (specify)
q17_1_uhc_attrib_other_txt	17.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	17.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q17_2_new_rooms_purpose	17.2. What are the rooms being used for?	17.2. What are the rooms being used for?			C. Universal Health Care (UHC) Implementation	alpha 240	
q18_inc_equipment	18. Has there been an increase in equipment in this facility?	18. Has there been an increase in equipment in this facility?		= No (2) → Q19	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q18_1_uhc_attrib	18.1. If yes, was it a result of the UHC Act enacted in 2019?	18.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequent enhanced or expanded due to UHC Act 3 = Newly implemented improved independent of UHC Act 4 = Not yet implemented but planned within the next 1-2 years 5 = Other (specify)
q18_1_uhc_attrib_other_txt	18.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	18.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q18_2_inc_equipment_list	18.2. If there was an increase in equipment, what are these pieces of equipment?	18.2. If there was an increase in equipment, what are these pieces of equipment?			C. Universal Health Care (UHC) Implementation	alpha 240	

q19_inc_supplies	19. Has there been an increase in supplies in this facility?	19. Has there been an increase in supplies in this facility?		= No (2) → Q20	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q19_1_uhc_attrib	19.1. If yes, was it a result of the UHC Act enacted in 2019?	19.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequently enhanced or expanded due to UHC Act 3 = Newly implemented improved independent of UHC Act 4 = Not yet implemented but planned within the next 1-2 years 5 = Other (specify)
q19_1_uhc_attrib_other_txt	19.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	19.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q19_2_inc_supplies_list	19.2. If there was an increase in supplies, what are these?	19.2. If there was an increase in supplies, what are these?			C. Universal Health Care (UHC) Implementation	alpha 240	
q20_emr_use	20. Have electronic medical records been used in this facility?	20. Have electronic medical records been used in this facility?		= No (2) → Q24 (skip Q21, Q22, Q23 — the DOH-IS fan)	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q20_1_uhc_attrib	20.1. If yes, was it a result of the UHC Act enacted in 2019?	20.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequently enhanced or expanded due to UHC Act 3 = Newly implemented improved independent of UHC Act 4 = Not yet implemented but planned within the next 1-2 years 5 = Other (specify)
q20_1_uhc_attrib_other_txt	20.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	20.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q21_data_submit	21. Does your facility currently submit health and financial data to the DOH Information System and/or the PhilHealth Dashboard?	21. Does your facility currently submit health and financial data to the DOH Information System and/or the PhilHealth Dashboard? READ OPTIONS OUT LOUD. SELECT ONE ANSWER ONLY.	not asked when Q20 EMR_USE = No (2)	= 4 (No, we are not submitting these data) → Q24 (skip Q22, Q23)	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes, to DOH Information System 2 = Yes, to PhilHealth Dashboard only 3 = Yes, to both DOH Information System and PhilHealth Dashboard 4 = No, we are not submitting these data 5 = Other (specify)
q21_other_txt	21. Does your facility currently submit health and financial data to the DOH Information System and/or the PhilHealth Dashboard? — Other (specify) text	21. Does your facility currently submit health and financial data to the DOH Information System and/or the PhilHealth Dashboard? — Other (specify) text	not asked when Q20 EMR_USE = No (2)		C. Universal Health Care (UHC) Implementation	alpha 120	
q22_data_freq	22. If yes, how frequently has your facility submit these data?	22. If yes, how frequently has your facility submit these data?	not asked when Q20 EMR_USE = No (2) not asked when Q21 DATA_SUBMIT = 4 (No, we are not submitting these data)		C. Universal Health Care (UHC) Implementation	numeric 1	1 = Weekly 2 = Monthly 3 = Quarterly 4 = Semi-annually 5 = Annually 6 = Other (specify)
q22_other_txt	22. If yes, how frequently has your facility submit these data? — Other (specify) text	22. If yes, how frequently has your facility submit these data? — Other (specify) text	not asked when Q20 EMR_USE = No (2) not asked when Q21 DATA_SUBMIT = 4 (No, we are not submitting these data)		C. Universal Health Care (UHC) Implementation	alpha 120	
q23_data_reports_used	23. Which of the submitted reports are actually used for decision-making?	23. Which of the submitted reports are actually used for decision-making? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.	not asked when Q20 EMR_USE = No (2) not asked when Q21 DATA_SUBMIT = 4 (No, we are not submitting these data)		C. Universal Health Care (UHC) Implementation	alpha 24	01 = OPD/IPD census 02 = Maternal, newborn child, and adolescent health (MNCAH) rep 03 = Notifiable disease

							surveillance reports 04 = Expenditure and budget utilization reports 05 = PhilHealth claim reimbursement reports 06 = YAKAP/Konsulta utilization reports 07 = NBB compliance reports 08 = ZBB compliance monitoring reports 09 = HRH staffing and deployment reports 10 = Medicines availability and stock status reports 11 = Facility performance scorecards / quality reports 99 = Other (specify)
q23_data_reports_used_other_txt	23. Which of the submitted reports are actually used for decision-making? — Other (specify) text	23. Which of the submitted reports are actually used for decision-making? — Other (specify) text	not asked when Q20 EMR_USE = No (2) not asked when Q21 DATA_SUBMIT = 4 (No, we are not submitting these data)		C. Universal Health Care (UHC) Implementation	alpha 120	
q24_staffing_changed	24. Have there been changes in the facility staffing?	24. Have there been changes in the facility staffing?		= No (2) → Q25	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q24_1_uhc_attrib	24.1. If yes, was it a result of the UHC Act enacted in 2019?	24.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequently enhanced or expanded due to UHC Act 3 = Newly implemented improved independent UHC Act 4 = Not yet implemented but planned within the next 1-2 years 5 = Other (specify)
q24_1_uhc_attrib_other_txt	24.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	24.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q25_referral_changed	25. Have there been changes in the referral system in this facility?	25. Have there been changes in the referral system in this facility?		= No (2) → Q26	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q25_1_uhc_attrib	25.1. If yes, was it a result of the UHC Act enacted in 2019?	25.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequently enhanced or expanded due to UHC Act 3 = Newly implemented improved independent UHC Act 4 = Not yet implemented but planned within the next 1-2 years 5 = Other (specify)
q25_1_uhc_attrib_other_txt	25.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	25.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q26_mou_moa	26. Does this facility has MoU/MoA with other health facilities apart of the healthcare provider network?	26. Does this facility has MoU/MoA with other health facilities apart of the healthcare provider network?		= No (2) → Q27	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q26_1_uhc_attrib	26.1. If yes, was it a result of the UHC Act enacted in 2019?	26.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequently enhanced or expanded due to UHC Act 3 = Newly implemented improved independent UHC Act 4 = Not yet implemented but planned within the next 1-2 years 5 = Other (specify)

							next 1-2 years 5 = Other (specify)
q26_1_uhc_attrb_other_txt	26.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	26.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q27_nbb	27. Does this facility implement No Balance Billing (NBB)?	27. Does this facility implement No Balance Billing (NBB)?		= No (2) → Q28	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q27_1_uhc_attrb	27.1. If yes, was it a result of the UHC Act enacted in 2019?	27.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequent enhanced or expanded due to UHC Act 3 = Newly implemented improved independent UHC Act 5 = Other (specify)
q27_1_uhc_attrb_other_txt	27.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	27.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q28_zbb	28. Does this facility implement Zero Balance Billing (ZBB)?	28. Does this facility implement Zero Balance Billing (ZBB)?		= No (2) → Q29	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q28_1_uhc_attrb	28.1. If yes, was it a result of the UHC Act enacted in 2019?	28.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequent enhanced or expanded due to UHC Act 3 = Newly implemented improved independent UHC Act 5 = Other (specify)
q28_1_uhc_attrb_other_txt	28.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	28.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q29_no_copay	29. Does this facility implement no co-payment policy?	29. Does this facility implement no co-payment policy?		= No (2) → Q30	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q29_1_uhc_attrb	29.1. If yes, was it a result of the UHC Act enacted in 2019?	29.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequent enhanced or expanded due to UHC Act 3 = Newly implemented improved independent UHC Act 5 = Other (specify)
q29_1_uhc_attrb_other_txt	29.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	29.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q30_ward_alloc	30. Does this facility implement ward accommodation allocation?	30. Does this facility implement ward accommodation allocation?		= No (2) → Q31	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q30_1_uhc_attrb	30.1. If yes, was it a result of the UHC Act enacted in 2019? (Under UHC, basic ward allocation is as follows: 90% for government general hospitals; 70% for government specialty hospitals, and 10% for private hospitals.)	30.1. If yes, was it a result of the UHC Act enacted in 2019? (Under UHC, basic ward allocation is as follows: 90% for government general hospitals; 70% for government specialty hospitals, and 10% for private hospitals.) SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequent enhanced or expanded due to UHC Act 3 = Newly implemented improved independent UHC Act 4 = Not yet implemented but planned within the next 1-2 years 5 = Other (specify)

q30_1_uhc_attrb_other_txt	30.1. If yes, was it a result of the UHC Act enacted in 2019? (Under UHC, basic ward allocation is as follows: 90% for government general hospitals; 70% for government specialty hospitals, and 10% for private hospitals.) — Other (specify) text	30.1. If yes, was it a result of the UHC Act enacted in 2019? (Under UHC, basic ward allocation is as follows: 90% for government general hospitals; 70% for government specialty hospitals, and 10% for private hospitals.) — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q31_cpg	31. Has there been an improvement in the clinical practice guidelines of this facility?	31. Has there been an improvement in the clinical practice guidelines of this facility?		= No (2) → Q32	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q31_1_uhc_attrb	31.1. If yes, was it a result of the UHC Act enacted in 2019?	31.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequent enhanced or expanded due to UHC Act 3 = Newly implemented improved independent UHC Act 4 = Not yet implemented but planned within the next 1-2 years 5 = Other (specify)
q31_1_uhc_attrb_other_txt	31.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	31.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q32_doh_lic_std	32. Does this facility implement DOH licensing standards?	32. Does this facility implement DOH licensing standards?		= No (2) → Q33	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q32_1_uhc_attrb	32.1. If yes, was it a result of the UHC Act enacted in 2019?	32.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequent enhanced or expanded due to UHC Act 3 = Newly implemented improved independent UHC Act 4 = Not yet implemented but planned within the next 1-2 years 5 = Other (specify)
q32_1_uhc_attrb_other_txt	32.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	32.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q33_phic_accred	33. Does this facility implement PhilHealth accreditation requirements?	33. Does this facility implement PhilHealth accreditation requirements?		= No (2) → Q34	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q33_1_uhc_attrb	33.1. If yes, was it a result of the UHC Act enacted in 2019?	33.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequent enhanced or expanded due to UHC Act 3 = Newly implemented improved independent UHC Act 4 = Not yet implemented but planned within the next 1-2 years 5 = Other (specify)
q33_1_uhc_attrb_other_txt	33.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	33.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q34_svc_delivery_prot	34. Does this facility implement service delivery protocols?	34. Does this facility implement service delivery protocols?		= No (2) → Q35	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No

q34_1_uhc_attrib	34.1. If yes, was it a result of the UHC Act enacted in 2019?	34.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequent enhanced or expanded due to UHC Act 3 = Newly implemented improved independent of UHC Act 4 = Not yet implemented but planned within the next 1-2 years 5 = Other (specify)
q34_1_uhc_attrib_other_txt	34.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	34.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q35_pcqm	35. Does this facility implement primary care quality measures?	35. Does this facility implement primary care quality measures?		= No (2) → Q36	C. Universal Health Care (UHC) Implementation	numeric 1	1 = Yes 2 = No
q35_1_uhc_attrib	35.1. If yes, was it a result of the UHC Act enacted in 2019?	35.1. If yes, was it a result of the UHC Act enacted in 2019? SELECT ONE ANSWER ONLY.			C. Universal Health Care (UHC) Implementation	numeric 1	1 = Implemented as direct result of the UHC Act 2 = Pre-existing prior UHC but subsequent enhanced or expanded due to UHC Act 3 = Newly implemented improved independent of UHC Act 4 = Not yet implemented but planned within the next 1-2 years 5 = Other (specify)
q35_1_uhc_attrib_other_txt	35.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text	35.1. If yes, was it a result of the UHC Act enacted in 2019? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q35_2_pcqm_measures	35.2. If yes, what are the primary care quality measures are you implementing?	35.2. If yes, what are the primary care quality measures are you implementing?			C. Universal Health Care (UHC) Implementation	alpha 6	01 = Client satisfaction survey 02 = Dashboards 99 = Other (specify)
q35_2_pcqm_measures_other_txt	35.2. If yes, what are the primary care quality measures are you implementing? — Other (specify) text	35.2. If yes, what are the primary care quality measures are you implementing? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q36_quality_chall	36. What are the major challenges to improving the quality of patient care in your local area?	36. What are the major challenges to improving the quality of patient care in your local area? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THE ANSWER OPTIONS THAT THE RESPONDENT GIVES WITHOUT PROMPTING.			C. Universal Health Care (UHC) Implementation	alpha 20	01 = Limited resources (e.g., shortages in healthcare personnel/manpower, medical equipment, essential supplies, or funding) 02 = Challenging quality standards (e.g., high standards that are difficult to achieve) 03 = Certain healthcare decisions are made by local government and not the health facility 04 = Lack of specific healthcare skills (i.e. Insufficient specialist surgeons, or other healthcare professionals with specialized skill needed in the facility) 05 = Inadequate training of healthcare workers (i.e., Healthcare workers needing additional training to meet the facility's quality standards) 06 = Lack of patient awareness of the benefits of UHC (e.g., patient not know they can access free consultations, a selected medicines and laboratory services) 07 = Limited accessibility of public healthcare facilities (e.g., lack of

							transportation options inconvenient location physical barriers hindering access for patients.) 08 = Infrastructure not conducive for patients (e.g., no ground floor shortage of rooms, inadequate sanitation facilities, lack of wheelchair accessibility etc.) 99 = Other (specify)
q36_quality_chall_other_txt	36. What are the major challenges to improving the quality of patient care in your local area? — Other (specify) text	36. What are the major challenges to improving the quality of patient care in your local area? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q37_access_chall	37. What are the major challenges to improving the accessibility of patient care in your local area?	37. What are the major challenges to improving the accessibility of patient care in your local area? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THE ANSWER OPTIONS THAT THE RESPONDENT GIVES WITHOUT PROMPTING.			C. Universal Health Care (UHC) Implementation	alpha 20	01 = Limited resources (e.g., shortages in healthcare personnel/manpower, medical equipment, essential supplies, or funding) 02 = Challenging quality standards (e.g., high standards that are difficult to achieve) 03 = Certain healthcare decisions are made by local government and not the health facility 04 = Lack of specific healthcare skills (i.e. Insufficient specialist surgeons, or other healthcare professionals with specialized skill needed in the facility) 05 = Inadequate training of healthcare workers (i.e., Healthcare workers needing additional training to meet the facility's quality standards) 06 = Lack of patient awareness of the benefits of UHC (e.g., patient not knowing they can access free consultations, or selected medicines and laboratory services) 07 = Limited accessibility of public healthcare facilities (e.g., lack of transportation options, inconvenient location, physical barriers hindering access for patients.) 08 = Infrastructure not conducive for patients (e.g., no ground floor, shortage of rooms, inadequate sanitation facilities, lack of wheelchair accessibility etc.) 99 = Other (specify)
q37_access_chall_other_txt	37. What are the major challenges to improving the accessibility of patient care in your local area? — Other (specify) text	37. What are the major challenges to improving the accessibility of patient care in your local area? — Other (specify) text			C. Universal Health Care (UHC) Implementation	alpha 120	
q38_yk_accred	38. Is the facility currently an accredited YAKAP/Konsulta provider?	The next questions we will ask relate to the YAKAP/Konsulta package. Please answer to the best of your knowledge. You may answer "I don't know" if you are unsure. 38. Is the facility currently an accredited YAKAP/Konsulta provider?		= No (2) → Q66 (skip Q39–Q65, the accredited YAKAP block)	D. YAKAP/Konsulta Package	numeric 1	1 = Yes 2 = No
q39_yk_since_month	39. If yes, since when? Month	39. If yes, since when? Month	not asked when Q38 YK_ACCRED = No (2)		D. YAKAP/Konsulta Package	numeric 2	

q39_yk_since_year	39. If yes, since when? Year	39. If yes, since when? Year	not asked when Q38 YK_ACCRED = No (2)		D. YAKAP/Konsulta Package	numeric 4	
q40_yk_package	40. If accredited, which of the following are included in the YAKAP/Konsulta package?	40. If accredited, which of the following are included in the YAKAP/Konsulta package? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.			D. YAKAP/Konsulta Package	alpha 20	01 = Pap smear 02 = Mammogram 03 = Lipid profile 04 = Thyroid functio 05 = Chest X-ray 06 = Low-dose CT sc 07 = Dental services 08 = All of the above 99 = Other (specify)
q40_yk_package_other_txt	40. If accredited, which of the following are included in the YAKAP/Konsulta package? — Other (specify) text	40. If accredited, which of the following are included in the YAKAP/Konsulta package? — Other (specify) text			D. YAKAP/Konsulta Package	alpha 120	
q41_yk_reg_indiv	41. Is it possible to register individual patients to YAKAP/Konsulta at this facility?	41. Is it possible to register individual patients to YAKAP/Konsulta at this facility?			D. YAKAP/Konsulta Package	numeric 1	1 = Yes 2 = No
q42_yk_reg_fam	42. Is it possible to register whole families to YAKAP/Konsulta at this facility?	42. Is it possible to register whole families to YAKAP/Konsulta at this facility?			D. YAKAP/Konsulta Package	numeric 1	1 = Yes 2 = No
q43_yk_reg_both	43. Is it possible to register both individual patients and their family members together to YAKAP/Konsulta at this facility?	43. Is it possible to register both individual patients and their family members together to YAKAP/Konsulta at this facility?			D. YAKAP/Konsulta Package	numeric 1	1 = Yes 2 = No
q44_capitation_amt	44. Based on your knowledge, what is the capitation amount of the YAKAP/Konsulta package?	44. Based on your knowledge, what is the capitation amount of the YAKAP/Konsulta package? Capitation is the amount per year per registered patient for delivering the YAKAP/Konsulta package services.			D. YAKAP/Konsulta Package	numeric 6	
q45_perf_indicators	45. What are the performance indicators you need to meet to receive the second tranche payment?	45. What are the performance indicators you need to meet to receive the second tranche payment? Performance indicators are the defined set of healthcare goods and services to be provided for each enrolled patient to receive the full capitation payment. DO NOT READ OPTIONS OUT LOUD. SELECT ALL THE ANSWER OPTIONS THAT THE RESPONDENT GIVES WITHOUT PROMPTING. RESPONDENT MAY NOT BE ALLOWED TO LOOK FOR REFERENCE PRIOR TO ANSWERING THIS QUESTION.			D. YAKAP/Konsulta Package	alpha 16	01 = Beneficiaries consulted a primary doctor 02 = Utilization of laboratory services 03 = Beneficiaries re antibiotics as prescri by their primary car doctor 04 = Beneficiaries re noncommunicable d (NCD) medicine as prescribed by their primary care doctor 05 = No requiremen 06 = 1st patient enc 99 = Other (specify)
q45_perf_indicators_other_txt	45. What are the performance indicators you need to meet to receive the second tranche payment? — Other (specify) text	45. What are the performance indicators you need to meet to receive the second tranche payment? — Other (specify) text			D. YAKAP/Konsulta Package	alpha 120	
q46_know_pay_freq	46. Do you know how often you can expect to receive payments from PhilHealth for the delivery of the YAKAP/Konsulta package?	46. Do you know how often you can expect to receive payments from PhilHealth for the delivery of the YAKAP/Konsulta package?		= No (2) → Q48 (skip Q47)	D. YAKAP/Konsulta Package	numeric 1	1 = Yes 2 = No
q47_pay_freq	47. How often should you be receiving payments?	47. How often should you be receiving payments? READ OPTIONS OUT LOUD. SELECT ONE ANSWER ONLY.	not asked when Q46 KNOW_PAY_FREQ = No (2)		D. YAKAP/Konsulta Package	numeric 1	1 = Monthly 2 = Quarterly 3 = Semi-annually 4 = Annually
q48_tranche_delay	48. Were there delays in receiving capitation tranches?	48. Were there delays in receiving capitation tranches?		= No (2) → Q49 — lands on the '_NUM' half of the hybrid pair, 'Q49_TRANCHE_INTERVAL_NUM'	D. YAKAP/Konsulta Package	numeric 1	1 = Yes 2 = No
q48_1_delay_reason	48.1. If yes, what was/were the reasons for the delay?	48.1. If yes, what was/were the reasons for the delay?			D. YAKAP/Konsulta Package	alpha 240	

q49_tranche_interval_num	49. On average, how long is the typical time interval between tranches releases to the facility? — No. of Days	49. On average, how long is the typical time interval between tranches releases to the facility? — No. of Days			D. YAKAP/Konsulta Package	numeric 4	
q49_tranche_interval	49. On average, how long is the typical time interval between tranches releases to the facility?	49. On average, how long is the typical time interval between tranches releases to the facility?			D. YAKAP/Konsulta Package	numeric 1	1 = 30 days and less 2 = 31-60 days 3 = More than 60 da
q50_accred_wait_num	50. How many days did you wait from application submission to accreditation approval? — No. of Days	50. How many days did you wait from application submission to accreditation approval? — No. of Days			D. YAKAP/Konsulta Package	numeric 4	
q50_accred_wait	50. How many days did you wait from application submission to accreditation approval?	50. How many days did you wait from application submission to accreditation approval?			D. YAKAP/Konsulta Package	numeric 1	1 = 30 days and less 2 = 31-60 days 3 = More than 60 da
q51_apply_reason	51. Why did you apply to become a YAKAP/Konsulta provider?	51. Why did you apply to become a YAKAP/Konsulta provider? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.			D. YAKAP/Konsulta Package	alpha 12	01 = Incentives (i.e., facility receives capitation/payment registered patients) 02 = Aligns with facility mission (i.e., goals o are aligned with the facility) 03 = Encouraged by 04 = Mandated/req by DOH/UHC 05 = To improve the services of the facilit 99 = Other (specify)
q51_apply_reason_other_txt	51. Why did you apply to become a YAKAP/Konsulta provider? — Other (specify) text	51. Why did you apply to become a YAKAP/Konsulta provider? — Other (specify) text			D. YAKAP/Konsulta Package	alpha 120	
q52_accred_difficult	52. Which of the following requirements were difficult to comply with for accreditation?	52. Which of the following requirements were difficult to comply with for accreditation? These are the requirements for YAKAP/Konsulta accreditation outlined by DOH. READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		= "None of the above" (90) only → Q62 — reached by falling through the whole Q53–Q61 gate chain, not by a single jump (see below)	D. YAKAP/Konsulta Package	alpha 20	01 = Ability to conduct preventive/screening services and health education 02 = Capability to provide services for required laboratory and radiologic services 03 = Capability to dispense required medicines 04 = General Infrastructure 05 = Equipment and Supplies 06 = Human resources 07 = Functional Health Information System 08 = Documentary requirements 09 = DOH licensing requirements 90 = None of the above
q53_why_diff_preventive	53. Why was it difficult to comply with: Ability to conduct preventive/screening services and health education?	53. Why was it difficult to comply with: Ability to conduct preventive/screening services and health education? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q52 option 01 not ticked → Q54	D. YAKAP/Konsulta Package	alpha 14	01 = Not enough but too expensive 02 = Time-consumin 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standard 99 = Other (specify)
q53_why_diff_preventive_other_txt	53. Why was it difficult to comply with: Ability to conduct preventive/screening services and health education? — Other (specify) text	53. Why was it difficult to comply with: Ability to conduct preventive/screening services and health education? — Other (specify) text			D. YAKAP/Konsulta Package	alpha 120	

q54_why_diff_lab	54. Why was it difficult to comply with: Capability to provide services for required laboratory and radiologic services?	54. Why was it difficult to comply with: Capability to provide services for required laboratory and radiologic services? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q52 option 02 not ticked → Q55	D. YAKAP/Konsulta Package	alpha 14	01 = Not enough but too expensive 02 = Time-consuming 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standards 99 = Other (specify)
q54_why_diff_lab_other_txt	54. Why was it difficult to comply with: Capability to provide services for required laboratory and radiologic services? — Other (specify) text	54. Why was it difficult to comply with: Capability to provide services for required laboratory and radiologic services? — Other (specify) text			D. YAKAP/Konsulta Package	alpha 120	
q55_why_diff_meds	55. Why was it difficult to comply with: Capability to dispense required medicines?	55. Why was it difficult to comply with: Capability to dispense required medicines? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q52 option 03 not ticked → Q56	D. YAKAP/Konsulta Package	alpha 14	01 = Not enough but too expensive 02 = Time-consuming 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standards 99 = Other (specify)
q55_why_diff_meds_other_txt	55. Why was it difficult to comply with: Capability to dispense required medicines? — Other (specify) text	55. Why was it difficult to comply with: Capability to dispense required medicines? — Other (specify) text			D. YAKAP/Konsulta Package	alpha 120	
q56_why_diff_infra	56. Why was it difficult to comply with: General Infrastructure?	56. Why was it difficult to comply with: General Infrastructure? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q52 option 04 not ticked → Q57	D. YAKAP/Konsulta Package	alpha 14	01 = Not enough but too expensive 02 = Time-consuming 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standards 99 = Other (specify)
q56_why_diff_infra_other_txt	56. Why was it difficult to comply with: General Infrastructure? — Other (specify) text	56. Why was it difficult to comply with: General Infrastructure? — Other (specify) text			D. YAKAP/Konsulta Package	alpha 120	
q57_why_diff_equipment	57. Why was it difficult to comply with: Equipment and Supplies?	57. Why was it difficult to comply with: Equipment and Supplies? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q52 option 05 not ticked → Q58	D. YAKAP/Konsulta Package	alpha 14	01 = Not enough but too expensive 02 = Time-consuming 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standards 99 = Other (specify)
q57_why_diff_equipment_other_txt	57. Why was it difficult to comply with: Equipment and Supplies? — Other (specify) text	57. Why was it difficult to comply with: Equipment and Supplies? — Other (specify) text			D. YAKAP/Konsulta Package	alpha 120	
q58_why_diff_hr	58. Why was it difficult to comply with: Human resource?	58. Why was it difficult to comply with: Human resource? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q52 option 06 not ticked → Q59	D. YAKAP/Konsulta Package	alpha 14	01 = Not enough but too expensive 02 = Time-consuming 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standards 99 = Other (specify)
q58_why_diff_hr_other_txt	58. Why was it difficult to comply with: Human resource? — Other (specify) text	58. Why was it difficult to comply with: Human resource? — Other (specify) text			D. YAKAP/Konsulta Package	alpha 120	
q59_why_diff_his	59. Why was it difficult to comply	59. Why was it difficult to comply with: Functional Health Information System?		Q52 option 07 not ticked → Q60	D. YAKAP/Konsulta Package	alpha 14	01 = Not enough but too expensive

	with: Functional Health Information System?	DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.			Package		02 = Time-consuming 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standards 99 = Other (specify)
q59_why_diff_his_other_txt	59. Why was it difficult to comply with: Functional Health Information System? — Other (specify) text	59. Why was it difficult to comply with: Functional Health Information System? — Other (specify) text			D. YAKAP/Konsulta Package	alpha 120	
q60_why_diff_docs	60. Why was it difficult to comply with: Documentary requirements?	60. Why was it difficult to comply with: Documentary requirements? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q52 option 08 not ticked → Q61	D. YAKAP/Konsulta Package	alpha 14	01 = Not enough budget 02 = Time-consuming 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standards 99 = Other (specify)
q60_why_diff_docs_other_txt	60. Why was it difficult to comply with: Documentary requirements? — Other (specify) text	60. Why was it difficult to comply with: Documentary requirements? — Other (specify) text			D. YAKAP/Konsulta Package	alpha 120	
q61_why_diff_doh_lic	61. Why was it difficult to comply with: DOH Licensing requirements?	61. Why was it difficult to comply with: DOH Licensing requirements? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q52 option 09 not ticked → Q62	D. YAKAP/Konsulta Package	alpha 14	01 = Not enough budget 02 = Time-consuming 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standards 99 = Other (specify)
q61_why_diff_doh_lic_other_txt	61. Why was it difficult to comply with: DOH Licensing requirements? — Other (specify) text	61. Why was it difficult to comply with: DOH Licensing requirements? — Other (specify) text			D. YAKAP/Konsulta Package	alpha 120	
q62_enroll_responsibility	62. Based on your understanding, whose responsibility is it to enroll patients to YAKAP/Konsulta?	62. Based on your understanding, whose responsibility is it to enroll patients to YAKAP/Konsulta? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.			D. YAKAP/Konsulta Package	alpha 14	01 = Patients' own initiative 02 = Facility 03 = LGU 04 = Someone else 05 = PhilHealth 99 = Other (specify)
q62_enroll_responsibility_other_txt	62. Based on your understanding, whose responsibility is it to enroll patients to YAKAP/Konsulta? — Other (specify) text	62. Based on your understanding, whose responsibility is it to enroll patients to YAKAP/Konsulta? — Other (specify) text			D. YAKAP/Konsulta Package	alpha 120	
q63_enroll_initiatives	63. Which of the following initiatives are you doing to enroll patients in this facility to YAKAP/Konsulta?	63. Which of the following initiatives are you doing to enroll patients in this facility to YAKAP/Konsulta? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.			D. YAKAP/Konsulta Package	alpha 18	01 = On-site Enrollment (e.g., offering patient enrollment at the health facility) 02 = LGU Outreach (involvement in LGU outreach activities, such as ongoing nutrition programs) 03 = Facility Outreach (e.g., engaging in outreach efforts directly from facility) 04 = Barangay Health Workers (BHWs) Support (e.g., receiving support from local BHWs) 05 = Information Campaigns (e.g., conducting information campaigns through various channels, including but not limited to online campaigns, house-to-house visits) 06 = Local Health

							Insurance Offices (L assistance or partne with YAKAP/Konsulta caravans 07 = Coordination w other government agencies and the pri sector 90 = No initiatives 99 = Other (specify)
q63_enroll_initiatives_other_txt	63. Which of the following initiatives are you doing to enroll patients in this facility to YAKAP/Konsulta? — Other (specify) text	63. Which of the following initiatives are you doing to enroll patients in this facility to YAKAP/Konsulta? — Other (specify) text			D. YAKAP/Konsulta Package	alpha 120	
q64_enroll_chall	64. Did you experience any challenges in enrolling patients to YAKAP/Konsulta?	64. Did you experience any challenges in enrolling patients to YAKAP/Konsulta?		= No (2) → Q72 (skip Q65)	D. YAKAP/Konsulta Package	numeric 1	1 = Yes 2 = No
q65_enroll_chall_list	65. What are the challenges you have faced?	65. What are the challenges you have faced? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.	not asked when Q38 YK_ACCRED = No (2) not asked when Q64 ENROLL_CHALL = No (2)	Q38 = Yes (accredited) → Q72 (skip Q66–Q71, the not-accredited block)	D. YAKAP/Konsulta Package	alpha 12	01 = Lack of patient awareness (i.e., pati are unaware of YAKAP/Konsulta, its benefits, and registr process) 02 = Lack of patient willingness (i.e., pati hesitant to provide personal informatio has concerns about security) 03 = Lack of resource (e.g., not enough manpower to condu information campai outreaches to enroll patients to YAKAP/Konsulta) 04 = Competition wi other health facilitie patient registration 05 = Technical / syst issues of PhilHealth data loss, errors, or platform accessibilit problems) 99 = Other (specify)
q65_enroll_chall_list_other_txt	65. What are the challenges you have faced? — Other (specify) text	65. What are the challenges you have faced? — Other (specify) text	not asked when Q38 YK_ACCRED = No (2) not asked when Q64 ENROLL_CHALL = No (2)		D. YAKAP/Konsulta Package	alpha 120	
q66_not_accred_reason	66. If not YAKAP/ Konsulta accredited, why are you not accredited?	66. If not YAKAP/ Konsulta accredited, why are you not accredited? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.	not asked when Q65 ENROLL_CHALL_LIST Q38 = Yes (accredited)	Q38 ≠ No (belt-and-braces entry gate) → Q72	D. YAKAP/Konsulta Package	alpha 8	01 = Difficult proces 02 = No time 03 = Ongoing applic 99 = Other (specify)
q66_not_accred_reason_other_txt	66. If not YAKAP/ Konsulta accredited, why are you not accredited? — Other (specify) text	66. If not YAKAP/ Konsulta accredited, why are you not accredited? — Other (specify) text	not asked when Q65 ENROLL_CHALL_LIST Q38 = Yes (accredited)		D. YAKAP/Konsulta Package	alpha 120	
q67_intend_accred	67. Are you intending to become a YAKAP/Konsulta provider?	67. Are you intending to become a YAKAP/Konsulta provider?		in 1, 2 (Yes, in process / Yes, not yet in process) → Q71 = 3 (No, decided not to) → Q69 = 4 (No, tried and failed) → Q70 = 5 (No, have not thought about it yet) → Q68 = 6 (I don't know) → Q79 — defect-fix: the paper sends this to Q72, which is the accredited-only block a Q67 respondent can never be in	D. YAKAP/Konsulta Package	numeric 1	1 = Yes, already in pr 2 = Yes, not yet in pr 3 = No, decided not 4 = No, tried and fail 5 = No, haven't thou about it yet
q68_know_how_start	68. If you decide to apply today, would you know how to start the process?	68. If you decide to apply today, would you know how to start the process?		(unconditional — the Q67 = 5 tail) → Q79	D. YAKAP/Konsulta Package	numeric 1	1 = Yes 2 = No
q69_decided_not_reason	69. What was the deciding factor not to apply?	69. What was the deciding factor not to apply?		(unconditional — the Q67 = 3 tail) → Q79	D. YAKAP/Konsulta Package	alpha 240	
q70_tried_failed_reason	70. What went wrong with the application?	70. What went wrong with the application?		(unconditional — the Q67 = 4 tail) → Q79	D. YAKAP/Konsulta Package	alpha 240	

q71_process_chall	71. What are some challenges in the process, if any?	71. What are some challenges in the process, if any?	not asked when Q65 ENROLL_CHALL_LIST Q38 = Yes (accredited)	(unconditional — the Q67 in 1,2 tail; the not-accredited block ends here) → Q79	D. YAKAP/Konsulta Package	alpha 240	
q72_catchment_area	72. What areas do you consider as the facility's catchment area/s?	72. What areas do you consider as the facility's catchment area/s?		Q38 = No (entry gate on the accredited costing block) → Q79	D. YAKAP/Konsulta Package	alpha 240	
q73_eligible_patients	73. How many patients in your catchment area are eligible to register to this YAKAP/Konsulta provider?	73. How many patients in your catchment area are eligible to register to this YAKAP/Konsulta provider?			D. YAKAP/Konsulta Package	numeric 7	
q74_registered_patients	74. How many eligible patients in your catchment area are already registered to this YAKAP/Konsulta provider?	74. How many eligible patients in your catchment area are already registered to this YAKAP/Konsulta provider?			D. YAKAP/Konsulta Package	numeric 7	
q75_is_1700_enough	75. The maximum per capita rate for YAKAP/Konsulta is Php 1,700 across private and public facilities (40% after first patient encounter, 60% based on registered catchment population by December). Based on your practice, is this enough?	75. The maximum per capita rate for YAKAP/Konsulta is Php 1,700 across private and public facilities (40% after first patient encounter, 60% based on registered catchment population by December). Based on your practice, is this enough?			D. YAKAP/Konsulta Package	numeric 1	1 = Yes 2 = No
q76_costing_done	76. Did you go through a costing exercise to figure out if this was viable for your facility?	76. Did you go through a costing exercise to figure out if this was viable for your facility?		= No (2) → Q78 (skip Q77)	D. YAKAP/Konsulta Package	numeric 1	1 = Yes 2 = No
q77_costing_viable	77. Did the costing exercise show that Php 1,700 was viable for your facility?	77. Did the costing exercise show that Php 1,700 was viable for your facility?	not asked when Q76 COSTING_DONE = No (2)	= No (2) → Q78 in 1, 3 (Yes / I don't know) → Q80	D. YAKAP/Konsulta Package	numeric 1	1 = Yes 2 = No
q78_min_cap_value_acc	78. What would be the minimum acceptable capitation value per patient per year for you as a YAKAP/Konsulta provider?	78. What would be the minimum acceptable capitation value per patient per year for you as a YAKAP/Konsulta provider?			D. YAKAP/Konsulta Package	numeric 6	
q79_min_cap_value_nonacc	79. What would be the minimum acceptable capitation value per patient per year for you to consider being a YAKAP/Konsulta provider?	79. What would be the minimum acceptable capitation value per patient per year for you to consider being a YAKAP/Konsulta provider?		Q38 ≠ No — entry gate; accredited facilities never see this item → Q80 (unconditional, once answered) → Q88 — the not-accredited arm exits Section D here	D. YAKAP/Konsulta Package	numeric 6	
q80_charge_add1_cap	80. Does your facility charge additional capitation fees?	80. Does your facility charge additional capitation fees?		= No (2) → Q82 (skip Q81)	D. YAKAP/Konsulta Package	numeric 1	1 = Yes 2 = No
q81_charge_add1_cap_reasons	81. What is/are the reason/s for the facility to charge additional capitation fees?	81. What is/are the reason/s for the facility to charge additional capitation fees? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.	not asked when Q80 CHARGE_ADD1_CAP = No (2)		D. YAKAP/Konsulta Package	alpha 12	01 = To cover expenses related to building maintenance, equipment and non-clinical staff 02 = A patient's care exceeds the predetermined fixed payment 03 = Services excluded from capitation coverage 04 = Provide preventive care that may not be adequately compensated under a basic capitation plan 05 = Offset losses 99 = Other (Specify)
q81_charge_add1_cap_reasons_other_txt	81. What is/are the reason/s for the facility to charge additional capitation fees? — Other (specify) text	81. What is/are the reason/s for the facility to charge additional capitation fees? — Other (specify) text	not asked when Q80 CHARGE_ADD1_CAP = No (2)		D. YAKAP/Konsulta Package	alpha 120	

q82_received_payments	82. Have you already received payments for patients enrolled?	82. Have you already received payments for patients enrolled? READ OPTIONS OUT LOUD. SELECT ONE ANSWER ONLY.		in 1, 2 (all expected payments / some but not all) → Q84 (skip Q83)	D. YAKAP/Konsulta Package	numeric 1	1 = Yes, we have received all expected payments yet 2 = Yes, we have received some but not all expected payments yet 3 = No, we have not received any expected payments yet 4 = No, we have not expected any payments yet
q83_not_received_reasons	83. Why not?	83. Why not? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.	not asked when Q82 RECEIVED_PAYMENTS in 1, 2 (all expected payments / some but not all)		D. YAKAP/Konsulta Package	alpha 14	01 = Delays in PhilHealth processing 02 = Delays in facility tracking of patient enrollment 03 = Difficulties in verifying patient enrollment (PhilHealth) 04 = Facility is not able in meeting criteria for payments (e.g., facility doesn't submit necessary requirements, facility doesn't enroll patients YAKAP/Konsulta) 05 = Criteria for payment is unclear 99 = Other (specify)
q83_not_received_reasons_other_txt	83. Why not? — Other (specify) text	83. Why not? — Other (specify) text	not asked when Q82 RECEIVED_PAYMENTS in 1, 2 (all expected payments / some but not all)		D. YAKAP/Konsulta Package	alpha 120	
q84_payment_chall	84. Did you face any challenges in getting these payments?	84. Did you face any challenges in getting these payments?		= No (2) → Q86 (skip Q85)	D. YAKAP/Konsulta Package	numeric 1	1 = Yes 2 = No
q85_payment_chall_list	85. What were these challenges?	85. What were these challenges? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.	not asked when Q84 PAYMENT_CHALL = No (2)		D. YAKAP/Konsulta Package	alpha 12	01 = Delayed payment process 02 = Unclear criteria for capitation 03 = Difficult to meet criteria for capitation 04 = PhilHealth process apply for payments is difficult/unclear 99 = Other (specify)
q85_payment_chall_list_other_txt	85. What were these challenges? — Other (specify) text	85. What were these challenges? — Other (specify) text	not asked when Q84 PAYMENT_CHALL = No (2)		D. YAKAP/Konsulta Package	alpha 120	
q86_expand_next	86. If you were to expand the YAKAP/Konsulta package, what would you expand next?	86. If you were to expand the YAKAP/Konsulta package, what would you expand next? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.			D. YAKAP/Konsulta Package	alpha 10	01 = The current list of medicines and drugs offered 02 = The current laboratory/diagnostic services offered 03 = Additional features 99 = Other (specify)
q86_expand_next_other_txt	86. If you were to expand the YAKAP/Konsulta package, what would you expand next? — Other (specify) text	86. If you were to expand the YAKAP/Konsulta package, what would you expand next? — Other (specify) text			D. YAKAP/Konsulta Package	alpha 120	
q87_addl_features	87. What additional features would you add?	87. What additional features would you add?			D. YAKAP/Konsulta Package	alpha 240	
q88_heard_bucas	88. Have you heard about the Bagong Urgent Care and Ambulatory Service (BUCAS)?	We will now ask questions related to BUCAS center and GAMOT package. 88. Have you heard about the Bagong Urgent Care and Ambulatory Service (BUCAS)?		= No (2) → Q95 (skip Q89–Q94, the whole BUCAS block)	E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	numeric 1	1 = Yes 2 = No
q89_has_bucas	89. Do you have a BUCAS Center?	89. Do you have a BUCAS Center?	not asked when Q88 HEARD_BUCAS = No (2)	= 1 (Yes) → Q91 (skip Q90) = 3 (I don't know) → Q95	E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	numeric 1	1 = Yes 2 = No
q90_no_bucas_reason	90. If none, what is the primary reason?	90. If none, what is the primary reason? READ OPTIONS OUT LOUD. SELECT ONE ANSWER ONLY.	not asked when Q89 HAS_BUCAS = 1 (Yes)	≠ 5 (anything but "Other") → Q95 — on 5 it falls through to the	E. Awareness on Expanded Health	numeric 2	1 = Proposal not yet submitted 2 = Limited information

				specify box first, which then resumes the same skip	Programs (BUCAS and GAMOT)		establishment process 3 = Did not meet state requirements 4 = Awaiting assessment or approval 5 = Others (specify)
q90_other_txt	90. If none, what is the primary reason? Other (specify)	90. If none, what is the primary reason? Other (specify)	not asked when Q89 HAS_BUCAS = 1 (Yes)		E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	alpha 120	
q91_bucas_services	91. What are the available services offered by your BUCAS Center?	91. What are the available services offered by your BUCAS Center? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.			E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	alpha 10	01 = Urgent care and consultation 02 = Minor surgical procedures 03 = Diagnostic and laboratory services 04 = Reproductive and special health services 99 = Other (specify)
q91_bucas_services_other_txt	91. What are the available services offered by your BUCAS Center? — Other (specify) text	91. What are the available services offered by your BUCAS Center? — Other (specify) text			E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	alpha 120	
q92_bucas_factors	92. In your assessment, what are the main factors affecting the utilization of BUCAS in your facility?	92. In your assessment, what are the main factors affecting the utilization of BUCAS in your facility? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.			E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	alpha 12	01 = Patient awareness 02 = Facility location accessibility 03 = Referral patterns 04 = PhilHealth coverage and reimbursement 05 = Availability of staff/services 99 = Others (specify)
q92_bucas_factors_other_txt	92. In your assessment, what are the main factors affecting the utilization of BUCAS in your facility? — Other (specify) text	92. In your assessment, what are the main factors affecting the utilization of BUCAS in your facility? — Other (specify) text			E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	alpha 120	
q93_bucas_resources_needed	93. What are the resources you need to support/sustain the BUCAS center?	93. What are the resources you need to support/sustain the BUCAS center?			E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	alpha 240	
q94_bucas_decongest	94. Based on your experience, does the BUCAS Center decongest your health facility of patients?	94. Based on your experience, does the BUCAS Center decongest your health facility of patients?	not asked when Q88 HEARD_BUCAS = No (2)	Q89 ≠ Yes (entry gate: Q91–Q94 are for facilities that HAVE a BUCAS Center) → Q95	E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	numeric 1	1 = Yes 2 = No
q95_heard_gamot	95. Have you heard about the Guaranteed and Accessible Medications for Outpatient Treatment (GAMOT) package?	95. Have you heard about the Guaranteed and Accessible Medications for Outpatient Treatment (GAMOT) package?		= No (2) → Q99 (skip Q96, Q97, Q98)	E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	numeric 1	1 = Yes 2 = No
q96_gamot_accred	96. Is your facility an accredited GAMOT provider?	96. Is your facility an accredited GAMOT provider?	not asked when Q95 HEARD_GAMOT = No (2)	= Yes (1) → Q98 (skip Q97)	E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	numeric 1	1 = Yes 2 = No
q97_no_gamot_reason	97. If no, what is the primary reason?	97. If no, what is the primary reason? READ OPTIONS OUT LOUD. SELECT ONE ANSWER ONLY.	not asked when Q95 HEARD_GAMOT = No (2) not asked when Q96 GAMOT_ACCRED = Yes (1)	≠ 5 (anything but "Other") → Q99 — same specify-box fall-through as Q90	E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	numeric 2	1 = Application not yet submitted 2 = Limited information 3 = Did not meet accreditation requirements 4 = Awaiting assessment or approval 5 = Others (specify)
q97_other_txt	97. If no, what is the primary reason? Other (specify)	97. If no, what is the primary reason? Other (specify)	not asked when Q95 HEARD_GAMOT = No (2) not asked when Q96 GAMOT_ACCRED = Yes (1)		E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	alpha 120	

q98_gamot_factors	98. In your assessment, what are the main factors affecting the utilization of the GAMOT Program in your facility?	98. In your assessment, what are the main factors affecting the utilization of the GAMOT Program in your facility? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.	not asked when Q95 HEARD_GAMOT = No (2)		E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	alpha 12	01 = Availability of GAMOT medicines 02 = Pharmacy capacity 03 = Patient awareness of the program 04 = PhilHealth eligibility and reimbursement processes 05 = Prescribing practices of physicians 99 = Others (specify)
q98_gamot_factors_other_txt	98. In your assessment, what are the main factors affecting the utilization of the GAMOT Program in your facility? — Other (specify) text	98. In your assessment, what are the main factors affecting the utilization of the GAMOT Program in your facility? — Other (specify) text	not asked when Q95 HEARD_GAMOT = No (2)		E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	alpha 120	
q99_stockout	99. In the past 3 months, has this facility experienced a stock-out (zero supply) of any tracer essential medicines?	99. In the past 3 months, has this facility experienced a stock-out (zero supply) of any tracer essential medicines?		= No (2) → Q105 (skip Q100–Q104, and with it the rest of Section E)	E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	numeric 1	1 = Yes 2 = No
q100_stockout_meds	100. What specific medicines? (antihypertensives, antibiotics, etc.)	100. What specific medicines? (antihypertensives, antibiotics, etc.)	not asked when Q99 STOCKOUT = No (2)		E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	alpha 240	
q101_stockout_duration	101. How many days did the stock-out last?	101. How many days did the stock-out last?			E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	numeric 2	1 = 30 days and less 2 = 31-60 days 3 = More than 60 days
q102_stockout_avg	102. On average, how many months do these stock-outs last?	102. On average, how many months do these stock-outs last?		Q101 ≠ 3 — entry gate; below 60 days the month band is implied by Q101 itself → Q103	E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	numeric 2	1 = less than a month 2 = 1-2 months 3 = 3-4 months 4 = 5-6 months 5 = more than 6 months
q103_addr_stockout	103. Did you do anything to address the medicine stock-outs in the GAMOT Package?	103. Did you do anything to address the medicine stock-outs in the GAMOT Package?		entry gate (#384): asked only when Q95 = Yes AND Q96 = Yes AND Q99 = Yes → Q105 in 2, 3 (No / Did not experience stock-outs under GAMOT) → Q105 (skip Q104)	E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	numeric 2	1 = Yes 2 = No 3 = Did not experience stock-outs of medicines under the GAMOT package
q104_addr_stockout_how	104. If yes, what did you do to address the medicine stock-outs in the GAMOT Package?	104. If yes, what did you do to address the medicine stock-outs in the GAMOT Package? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.	not asked when Q99 STOCKOUT = No (2) not asked when Q103 ADDR_STOCKOUT in 2, 3 (No / Did not experience stock-outs under GAMOT)		E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	alpha 8	01 = Resorted to alternative procurement 02 = Active inventory monitoring 03 = Improve forecasting and quantification 99 = Other (specify)
q104_addr_stockout_how_other_txt	104. If yes, what did you do to address the medicine stock-outs in the GAMOT Package? — Other (specify) text	104. If yes, what did you do to address the medicine stock-outs in the GAMOT Package? — Other (specify) text	not asked when Q99 STOCKOUT = No (2) not asked when Q103 ADDR_STOCKOUT in 2, 3 (No / Did not experience stock-outs under GAMOT)		E. Awareness on Expanded Health Programs (BUCAS and GAMOT)	alpha 120	
q105_doh_licensed	105. Is this facility DOH licensed?	We will now ask you about your facility's experience with the licensing process. 105. Is this facility DOH licensed?		in 2, 3, 4 (No / submitted and waiting / don't know what DOH licensing is) → Q122 (skip Q106–Q121, the whole of Section F)	F. DOH Licensing: Status and Barriers to Licensing	numeric 2	1 = Yes 2 = No 3 = No, but have submitted requirements and waiting for license
q106_lic_received_when	106. When did you receive your DOH license from your most recent application?	106. When did you receive your DOH license from your most recent application?	not asked when Q105 DOH_LICENSED in 2, 3, 4 (No / submitted and waiting / don't know what DOH licensing is)		F. DOH Licensing: Status and Barriers to Licensing	numeric 2	1 = Within the last 1 month 2 = Within the last 4 months 3 = Over 6 months but within 1 year 4 = More than 1 year
q107_lic_days_num	107. How many days did it take you to receive the license? — No. of Days	107. How many days did it take you to receive the license? — No. of Days			F. DOH Licensing: Status and Barriers to Licensing	numeric 4	
q107_lic_days	107. How many days did it take you to receive the license?	107. How many days did it take you to receive the license?			F. DOH Licensing: Status and Barriers to Licensing	numeric 2	1 = 30 days and less 2 = 31-60 days 3 = More than 60 days

q108_doh_lic_difficult	108. Which of the following requirements were difficult to comply with in the DOH licensing process?	108. Which of the following requirements were difficult to comply with in the DOH licensing process? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		= "None of the above" (90) only → Q122 — same cascade mechanism as Q52, via Q109–Q121	F. DOH Licensing: Status and Barriers to Licensing	alpha 28	01 = Patient rights and organization ethics 02 = Patient care 03 = Leadership and management 04 = Human resource management 05 = Information management 06 = Safe practice environment 07 = Improving performance 08 = Physical plant 09 = Equipment and instruments 10 = National laws and DOH issuances implemented in hospitals and other health facilities (hospitals only) 11 = Emergency cart contents (hospitals only) 12 = Add-on services (hospitals only) 13 = Public access to information (primary facilities only) 90 = None of the above
q109_why_diff_pt_rights	109. Why was it difficult to comply with: Patient rights and organization ethics?	109. Why was it difficult to comply with: Patient rights and organization ethics? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q108 option 01 not ticked → Q110	F. DOH Licensing: Status and Barriers to Licensing	alpha 18	01 = Not enough budget too expensive 02 = Time-consuming 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standards 07 = Lack of training 08 = Lack of space 99 = Other (specify)
q109_why_diff_pt_rights_other_txt	109. Why was it difficult to comply with: Patient rights and organization ethics? — Other (specify) text	109. Why was it difficult to comply with: Patient rights and organization ethics? — Other (specify) text			F. DOH Licensing: Status and Barriers to Licensing	alpha 120	
q110_why_diff_pt_care	110. Why was it difficult to comply with: Patient care?	110. Why was it difficult to comply with: Patient care? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q108 option 02 not ticked → Q111	F. DOH Licensing: Status and Barriers to Licensing	alpha 18	01 = Not enough budget too expensive 02 = Time-consuming 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standards 07 = Lack of training 08 = Lack of space 99 = Other (specify)
q110_why_diff_pt_care_other_txt	110. Why was it difficult to comply with: Patient care? — Other (specify) text	110. Why was it difficult to comply with: Patient care? — Other (specify) text			F. DOH Licensing: Status and Barriers to Licensing	alpha 120	
q111_why_diff_leadership	111. Why was it difficult to comply with: Leadership and management?	111. Why was it difficult to comply with: Leadership and management? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q108 option 03 not ticked → Q112	F. DOH Licensing: Status and Barriers to Licensing	alpha 22	01 = Not enough budget too expensive 02 = Time-consuming 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standards 07 = Lack of training 08 = Lack of space 09 = Frequent changing guidelines and policies 10 = Resistance to change of staff 99 = Other (specify)
q111_why_diff_leadership_other_txt	111. Why was it difficult to comply with: Leadership and management? — Other (specify) text	111. Why was it difficult to comply with: Leadership and management? — Other (specify) text			F. DOH Licensing: Status and Barriers to Licensing	alpha 120	

q112_why_diff_hrm	112. Why was it difficult to comply with: Human resource management?	112. Why was it difficult to comply with: Human resource management? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q108 option 04 not ticked → Q113	F. DOH Licensing: Status and Barriers to Licensing	alpha 20	01 = Not enough budget too expensive 02 = Time-consuming 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standards 07 = Lack of training 08 = Lack of space 09 = Staff are resistant to change 99 = Other (specify)
q112_why_diff_hrm_other_txt	112. Why was it difficult to comply with: Human resource management? — Other (specify) text	112. Why was it difficult to comply with: Human resource management? — Other (specify) text			F. DOH Licensing: Status and Barriers to Licensing	alpha 120	
q113_why_diff_info_mgmt	113. Why was it difficult to comply with: Information management?	113. Why was it difficult to comply with: Information management? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q108 option 05 not ticked → Q114	F. DOH Licensing: Status and Barriers to Licensing	alpha 18	01 = Not enough budget too expensive 02 = Time-consuming 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standards 07 = Lack of training 08 = Lack of space 99 = Other (specify)
q113_why_diff_info_mgmt_other_txt	113. Why was it difficult to comply with: Information management? — Other (specify) text	113. Why was it difficult to comply with: Information management? — Other (specify) text			F. DOH Licensing: Status and Barriers to Licensing	alpha 120	
q114_why_diff_safe	114. Why was it difficult to comply with: Safe practice and environment?	114. Why was it difficult to comply with: Safe practice and environment? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q108 option 06 not ticked → Q115	F. DOH Licensing: Status and Barriers to Licensing	alpha 18	01 = Not enough budget too expensive 02 = Time-consuming 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standards 07 = Lack of training 08 = Lack of space 99 = Other (specify)
q114_why_diff_safe_other_txt	114. Why was it difficult to comply with: Safe practice and environment? — Other (specify) text	114. Why was it difficult to comply with: Safe practice and environment? — Other (specify) text			F. DOH Licensing: Status and Barriers to Licensing	alpha 120	
q115_why_diff_perf	115. Why was it difficult to comply with: Improving performance?	115. Why was it difficult to comply with: Improving performance? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q108 option 07 not ticked → Q116	F. DOH Licensing: Status and Barriers to Licensing	alpha 18	01 = Not enough budget too expensive 02 = Time-consuming 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standards 07 = Lack of training 08 = Lack of space 99 = Other (specify)
q115_why_diff_perf_other_txt	115. Why was it difficult to comply with: Improving performance? — Other (specify) text	115. Why was it difficult to comply with: Improving performance? — Other (specify) text			F. DOH Licensing: Status and Barriers to Licensing	alpha 120	
q116_why_diff_phys_plant	116. Why was it difficult to comply with: Physical plant?	116. Why was it difficult to comply with: Physical plant? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q108 option 08 not ticked → Q117	F. DOH Licensing: Status and Barriers to Licensing	alpha 18	01 = Not enough budget too expensive 02 = Time-consuming 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standards 07 = Lack of training

							08 = Lack of space 99 = Other (specify)
q116_why_diff_phys_plant_other_txt	116. Why was it difficult to comply with: Physical plant? — Other (specify) text	116. Why was it difficult to comply with: Physical plant? — Other (specify) text			F. DOH Licensing: Status and Barriers to Licensing	alpha 120	
q117_why_diff_price_info	117. Why was it difficult to comply with: Public access to price information?	117. Why was it difficult to comply with: Public access to price information? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q108 option 13 not ticked (PCF-only topic — the printed tail is reordered vs the gate's codes) → Q118	F. DOH Licensing: Status and Barriers to Licensing	alpha 18	01 = Not enough budget too expensive 02 = Time-consuming 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standards 07 = Lack of training 08 = Lack of space 99 = Other (specify)
q117_why_diff_price_info_other_txt	117. Why was it difficult to comply with: Public access to price information? — Other (specify) text	117. Why was it difficult to comply with: Public access to price information? — Other (specify) text			F. DOH Licensing: Status and Barriers to Licensing	alpha 120	
q118_why_diff_equipment	118. Why was it difficult to comply with: Equipment and instruments?	118. Why was it difficult to comply with: Equipment and instruments? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q108 option 09 not ticked → Q119	F. DOH Licensing: Status and Barriers to Licensing	alpha 18	01 = Not enough budget too expensive 02 = Time-consuming 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standards 07 = Lack of training 08 = Lack of space 99 = Other (specify)
q118_why_diff_equipment_other_txt	118. Why was it difficult to comply with: Equipment and instruments? — Other (specify) text	118. Why was it difficult to comply with: Equipment and instruments? — Other (specify) text			F. DOH Licensing: Status and Barriers to Licensing	alpha 120	
q119_why_diff_nat_laws	119. Why was it difficult to comply with: National laws and DOH issuances implemented in hospitals and other health facilities?	119. Why was it difficult to comply with: National laws and DOH issuances implemented in hospitals and other health facilities? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q108 option 10 not ticked → Q120	F. DOH Licensing: Status and Barriers to Licensing	alpha 18	01 = Not enough budget too expensive 02 = Time-consuming 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standards 07 = Lack of training 08 = Lack of space 99 = Other (specify)
q119_why_diff_nat_laws_other_txt	119. Why was it difficult to comply with: National laws and DOH issuances implemented in hospitals and other health facilities? — Other (specify) text	119. Why was it difficult to comply with: National laws and DOH issuances implemented in hospitals and other health facilities? — Other (specify) text			F. DOH Licensing: Status and Barriers to Licensing	alpha 120	
q120_why_diff_emerg_cart	120. Why was it difficult to comply with: Emergency Cart Contents?	120. Why was it difficult to comply with: Emergency Cart Contents? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q108 option 11 not ticked (hospitals only) → Q121	F. DOH Licensing: Status and Barriers to Licensing	alpha 18	01 = Not enough budget too expensive 02 = Time-consuming 03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent standards 07 = Lack of training 08 = Lack of space 99 = Other (specify)
q120_why_diff_emerg_cart_other_txt	120. Why was it difficult to comply with: Emergency Cart Contents? — Other (specify) text	120. Why was it difficult to comply with: Emergency Cart Contents? — Other (specify) text			F. DOH Licensing: Status and Barriers to Licensing	alpha 120	
q121_why_diff_addons	121. Why was it difficult to comply	121. Why was it difficult to comply with: Add-on services? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.	not asked when Q105 DOH_LICENSED in 2, 3, 4 (No / submitted and	Q108 option 12 not ticked (hospitals only) → Q122	F. DOH Licensing: Status and	alpha 18	01 = Not enough budget too expensive 02 = Time-consuming

	with: Add-on services?		waiting / don't know what DOH licensing is)		Barriers to Licensing		03 = Limited human resources 04 = Legal processes 05 = Compiling documentary requirements 06 = Stringent stand 07 = Lack of training 08 = Lack of space 99 = Other (specify)
q121_why_diff_addons_other_txt	121. Why was it difficult to comply with: Add-on services? — Other (specify) text	121. Why was it difficult to comply with: Add-on services? — Other (specify) text	not asked when Q105 DOH_LICENSED in 2, 3, 4 (No / submitted and waiting / don't know what DOH licensing is)		F. DOH Licensing: Status and Barriers to Licensing	alpha 120	
q122_nbb_curr	122. Do you currently implement the No Balance Billing (NBB) for your patients?	The next set of questions covers the service delivery process. This section covers the implementation of NBB and ZBB, LGU support, and the Referral System. 122. Do you currently implement the No Balance Billing (NBB) for your patients?		= No (2) → Q125 (skip Q123, Q124)	G. Service Delivery Process	numeric 1	1 = Yes 2 = No
q123_nbb_all_patients	123. Are you able to implement NBB for all patients, to the best of your knowledge, for the last 6 months?	123. Are you able to implement NBB for all patients, to the best of your knowledge, for the last 6 months?	not asked when Q122 NBB_CURR = No (2)		G. Service Delivery Process	numeric 1	1 = Yes 2 = No
q124_nbb_barriers	124. In your view, what are some of the barriers to implementing NBB?	124. In your view, what are some of the barriers to implementing NBB? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.	not asked when Q122 NBB_CURR = No (2)		G. Service Delivery Process	alpha 16	01 = Complying with no fees for basic or v accommodation 02 = Complying the prescribed ratio of allocation of basic a non-basic accommo 03 = Patients do not through the process availing it 04 = Insufficient PhilHealth support v 05 = Insufficient oth sources (e.g. MAIFIP DSWD, PCSO) (late payments applicable MAIFIP) 06 = PhilHealth dela payment 90 = None of the ab 99 = Other (specify)
q124_nbb_barriers_other_txt	124. In your view, what are some of the barriers to implementing NBB? — Other (specify) text	124. In your view, what are some of the barriers to implementing NBB? — Other (specify) text	not asked when Q122 NBB_CURR = No (2)		G. Service Delivery Process	alpha 120	
q125_zbb_curr	125. Do you currently implement the Zero Balance Billing (ZBB) for your patients?	125. Do you currently implement the Zero Balance Billing (ZBB) for your patients?		= No (2) → Q128 (skip Q126, Q127)	G. Service Delivery Process	numeric 1	1 = Yes 2 = No
q126_zbb_all_patients	126. Are you able to implement ZBB for all patients, to the best of your knowledge, for the last six months?	126. Are you able to implement ZBB for all patients, to the best of your knowledge, for the last six months?	not asked when Q125 ZBB_CURR = No (2)		G. Service Delivery Process	numeric 1	1 = Yes 2 = No
q127_zbb_barriers	127. In your view, what are some of the barriers to implementing ZBB?	127. In your view, what are some of the barriers to implementing ZBB? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.	not asked when Q125 ZBB_CURR = No (2)		G. Service Delivery Process	alpha 16	01 = Complying with no fees for basic or v accommodation 02 = Complying the prescribed ratio of allocation of basic a non-basic accommo 03 = Patients do not through the process availing it 04 = Insufficient PhilHealth support v 05 = Insufficient oth sources (e.g. MAIFIP DSWD, PCSO) (late payments applicable MAIFIP) 06 = PhilHealth dela payment 90 = None of the ab 99 = Other (specify)

q127_zbb_barriers_other_txt	127. In your view, what are some of the barriers to implementing ZBB? — Other (specify) text	127. In your view, what are some of the barriers to implementing ZBB? — Other (specify) text	not asked when Q125 ZBB_CURR = No (2)		G. Service Delivery Process	alpha 120	
q128_allow_oop_basic	128. Does the facility allow out-of-pocket (OOP) expenses for basic accommodation?	128. Does the facility allow out-of-pocket (OOP) expenses for basic accommodation?		= No (2) → Q130 (skip Q129)	G. Service Delivery Process	numeric 1	1 = Yes 2 = No
q129_oop_reason	129. Why does the facility allow OOP expenses for basic accommodation? Specify the reason.	129. Why does the facility allow OOP expenses for basic accommodation? Specify the reason.	not asked when Q128 ALLOW_OOP_BASIC = No (2)		G. Service Delivery Process	alpha 240	
q130_difficult_benefit	130. Which of the UHC benefits do you find most difficult to implement?	130. Which of the UHC benefits do you find most difficult to implement?			G. Service Delivery Process	numeric 2	1 = PhilHealth/financial protection benefits 2 = Establishment of health care provider networks (HCPNs) (i.e. referral system) 3 = Human resource health reforms 4 = Other (specify)
q130_other_txt	130. Which of the UHC benefits do you find most difficult to implement? Other (specify)	130. Which of the UHC benefits do you find most difficult to implement? Other (specify)			G. Service Delivery Process	alpha 120	
q131_difficult_reason	131. Why is this difficult to implement?	131. Why is this difficult to implement?			G. Service Delivery Process	alpha 8	01 = The implementation of UHC benefits is highly reliant on LGU decisions 02 = Not enough funding/budget 03 = Technical/systemic issues of PhilHealth data loss, errors, or platform accessibility problems) 99 = Other (specify)
q131_difficult_reason_other_txt	131. Why is this difficult to implement? — Other (specify) text	131. Why is this difficult to implement? — Other (specify) text			G. Service Delivery Process	alpha 120	
q132_malasakit_provided	132. Has the facility been providing medical social welfare or assistance (e.g., through Malasakit Centers, MAIFIP)?	132. Has the facility been providing medical social welfare or assistance (e.g., through Malasakit Centers, MAIFIP)?		= No (2) → Q134 (skip Q133)	G. Service Delivery Process	numeric 1	1 = Yes 2 = No
q133_malasakit_why	133. Why is the facility providing medical social welfare or assistance through Malasakit Centers or MAIFIP?	133. Why is the facility providing medical social welfare or assistance through Malasakit Centers or MAIFIP? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.	not asked when Q132 MALASAKIT_PROVIDED = No (2)		G. Service Delivery Process	alpha 10	01 = Streamline access to medical and financial services for indigent and financially incapacitated patients 02 = Reduce out-of-pocket expenses 03 = Eliminate the need to travel to multiple government agencies 04 = Foster a more compassionate approach to healthcare 99 = Other (specify)
q133_malasakit_why_other_txt	133. Why is the facility providing medical social welfare or assistance through Malasakit Centers or MAIFIP? — Other (specify) text	133. Why is the facility providing medical social welfare or assistance through Malasakit Centers or MAIFIP? — Other (specify) text	not asked when Q132 MALASAKIT_PROVIDED = No (2)		G. Service Delivery Process	alpha 120	
q134_no_malasakit_why	134. Why is the facility not providing medical social welfare or assistance through Malasakit Centers or MAIFIP?	134. Why is the facility not providing medical social welfare or assistance through Malasakit Centers or MAIFIP? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.		Q132 = Yes — entry gate; the not-provided block does not apply → Q135	G. Service Delivery Process	alpha 10	01 = Limited budget 02 = Stringent eligibility requirements 03 = Incomplete documentation from patients 04 = High patient volume leading to service bottlenecks 99 = Other (specify)
q134_no_malasakit_why_other_txt	134. Why is the facility not providing	134. Why is the facility not providing medical social welfare or assistance			G. Service Delivery	alpha 120	

	medical social welfare or assistance through Malasakit Centers or MAIFIP? — Other (specify) text	through Malasakit Centers or MAIFIP? — Other (specify) text			Process		
q135_lgu_support	135. Do you receive any support from your LGU to implement UHC reforms?	135. Do you receive any support from your LGU to implement UHC reforms?		= No (2) → Q139 (skip Q136, Q137, Q138)	G. Service Delivery Process	numeric 1	1 = Yes 2 = No
q136_lgu_support_forms	136. What forms of support do you receive?	136. What forms of support do you receive? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.	not asked when Q135 LGU_SUPPORT = No (2)		G. Service Delivery Process	alpha 10	01 = Financial assistance 02 = Technical assistance 03 = Medical supplies/equipment 04 = Manpower support 99 = Other (specify)
q136_lgu_support_forms_other_txt	136. What forms of support do you receive? — Other (specify) text	136. What forms of support do you receive? — Other (specify) text	not asked when Q135 LGU_SUPPORT = No (2)		G. Service Delivery Process	alpha 120	
q137_lgu_satisfied	137. Are you satisfied with the support you receive from your LGU?	137. Are you satisfied with the support you receive from your LGU?	not asked when Q135 LGU_SUPPORT = No (2)	= Yes (1) → Q139 (skip Q138) — defect-fix: the paper sends Yes to Q141, which orphans the Q139/Q140 PHO protocol pair for every satisfied respondent	G. Service Delivery Process	numeric 1	1 = Yes 2 = No
q138_lgu_not_sat_why	138. Why not?	138. Why not? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.	not asked when Q135 LGU_SUPPORT = No (2) not asked when Q137 LGU_SATISFIED = Yes (1)		G. Service Delivery Process	alpha 10	01 = Insufficient 02 = Hard to coordinate 03 = Support given is not aligned with the need of the facility 99 = Other (specify)
q138_lgu_not_sat_why_other_txt	138. Why not? — Other (specify) text	138. Why not? — Other (specify) text	not asked when Q135 LGU_SUPPORT = No (2) not asked when Q137 LGU_SATISFIED = Yes (1)		G. Service Delivery Process	alpha 120	
q139_pho_protocol_clarity	139. How clear are the protocols regarding which decisions require Provincial Health Office (PHO) approval versus those you can decide at the facility level?	139. How clear are the protocols regarding which decisions require Provincial Health Office (PHO) approval versus those you can decide at the facility level?		entry gate: asked only when Q7 = Public AND Q8 in Level 1/2/3 Hospital (#386) → Q141 (skip Q140) in 1, 2 (Very Clear / Clear) → Q141 (skip Q140)	G. Service Delivery Process	numeric 2	1 = Very Clear 2 = Clear 3 = Neither 4 = Unclear 5 = Very unclear
q140_unclear_protocol	140. Which specific protocol that you consider as unclear?	140. Which specific protocol that you consider as unclear?	not asked when Q139 PHO_PROTOCOL_CLARITY entry gate: asked only when Q7 = Public AND Q8 in Level 1/2/3 Hospital (#386) not asked when Q139 PHO_PROTOCOL_CLARITY in 1, 2 (Very Clear / Clear)		G. Service Delivery Process	alpha 240	
q141_num_referred_out	141. In the past 6 months, how many patients were referred to a higher-level facility within the referral network?	141. In the past 6 months, how many patients were referred to a higher-level facility within the referral network?			G. Service Delivery Process	numeric 6	
q142_send_referral_how	142. What are the most common ways you send referrals to higher level facilities/specialists?	142. What are the most common ways you send referrals to higher level facilities/specialists? Our focus is specifically on referrals external to the facility, excluding internal referrals. READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.			G. Service Delivery Process	alpha 8	01 = Physical referral 02 = E-referral 03 = Referring facility receiving facility 99 = Other (specify)
q142_send_referral_how_other_txt	142. What are the most common ways you send referrals to higher level facilities/specialists? — Other (specify) text	142. What are the most common ways you send referrals to higher level facilities/specialists? — Other (specify) text			G. Service Delivery Process	alpha 120	
q143_referral_form_type	143. What type of referral form do you use to send to higher level facilities?	143. What type of referral form do you use to send to higher level facilities? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.			G. Service Delivery Process	alpha 12	01 = DOH standard referral form 02 = Facility's standard referral form 03 = Province's standard referral form 04 = City / LGU standard referral form 05 = No standard referral form

							form 99 = Other (specify)
q143_referral_form_type_other_txt	143. What type of referral form do you use to send to higher level facilities? — Other (specify) text	143. What type of referral form do you use to send to higher level facilities? — Other (specify) text			G. Service Delivery Process	alpha 120	
q144_specialist_network	144. Do you have a network of specialist providers to refer patients to, if needed?	144. Do you have a network of specialist providers to refer patients to, if needed? READ OPTIONS OUT LOUD. SELECT ONE ANSWER ONLY.			G. Service Delivery Process	numeric 2	1 = Yes 2 = No 3 = I've never heard
q145_ref_proportion	145. Considering all patients who come to your facility for the past 6 months, what is the proportion of patients referred by another facility compared to those who self-refer/walk-in?	145. Considering all patients who come to your facility for the past 6 months, what is the proportion of patients referred by another facility compared to those who self-refer/walk-in? READ OPTIONS OUT LOUD. SELECT ONE ANSWER ONLY.			G. Service Delivery Process	numeric 2	1 = Almost all patients referred, very few walk-in/self-referred 2 = Majority of patients are referred, some walk-in/self-referred 3 = The proportion of referrals is about equal 4 = Majority of patients walk-in/self-referred some are referred 5 = Almost all patients walk-in/self-referred few are referred 6 = I am unsure about typical ratio of referrals walk-ins
q146_receive_referral_how	146. Of those referred, which of the following are the most common ways you receive referrals from lower-level health facilities?	146. Of those referred, which of the following are the most common ways you receive referrals from lower-level health facilities? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.			G. Service Delivery Process	alpha 8	01 = Physical referral 02 = E-referral 03 = Referring facility receiving facility 99 = Other (specify)
q146_receive_referral_how_other_txt	146. Of those referred, which of the following are the most common ways you receive referrals from lower-level health facilities? — Other (specify) text	146. Of those referred, which of the following are the most common ways you receive referrals from lower-level health facilities? — Other (specify) text			G. Service Delivery Process	alpha 120	
q147_external_services_go	147. Where do your patients go to get the services not available at this facility?	147. Where do your patients go to get the services not available at this facility?			G. Service Delivery Process	alpha 10	01 = External laboratory 02 = Other private facility 03 = Other public facility (e.g., urban/rural health centers, barangay health centers, city/municipal health offices) 99 = Other, (specify)
q148_ref_satisfaction	148. How would you rate your satisfaction with your current referral system?	148. How would you rate your satisfaction with your current referral system? READ OPTIONS OUT LOUD. SELECT ONE ANSWER ONLY.		in 1, 2 (Very Satisfied / Satisfied) → Q150 (skip Q149)	G. Service Delivery Process	numeric 2	1 = Very Satisfied: No improvements need 'patients are always referred appropriate 2 = Satisfied: Minor improvements need patients are general referred appropriate 3 = Neither Satisfied Dissatisfied: Improvements need but generally functional 4 = Dissatisfied: Major improvements need number of patients referred to the wrong specialists or do not receive appropriate follow-up care 5 = Very Dissatisfied Major improvement needed, many patients are referred to the wrong specialists or do not receive appropriate follow-up care
q149_not_satisfied_why	149. Why are you not satisfied with the current referral system?	149. Why are you not satisfied with the current referral system? DO NOT READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.	not asked when Q148 REF_SATISFACTION in 1, 2 (Very Satisfied / Satisfied)		G. Service Delivery Process	alpha 8	01 = Facilities are overcrowded/overcrowded and do not accept other patient referrals 02 = The referral process is slow 03 = There is poor coordination between

							facility and referred facilities 99 = Other (specify)
q149_not_satisfied_why_other_txt	149. Why are you not satisfied with the current referral system? — Other (specify) text	149. Why are you not satisfied with the current referral system? — Other (specify) text	not asked when Q148 REF_SATISFACTION in 1, 2 (Very Satisfied / Satisfied)		G. Service Delivery Process	alpha 120	
q150_hr_chall	150. What challenges in human resources do you have?	We will now proceed to the last section of this survey, which focuses on human resources. 150. What challenges in human resources do you have? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.			H. Human Resources for Health	alpha 10	01 = Understaffing 02 = Skills mismatch of skills 03 = Retention / high turnover 04 = Multi-tasking 99 = Other (specify)
q150_hr_chall_other_txt	150. What challenges in human resources do you have? — Other (specify) text	150. What challenges in human resources do you have? — Other (specify) text			H. Human Resources for Health	alpha 120	
q151_improvement_area	151. What area do you find the most room for improvement in your staff?	151. What area do you find the most room for improvement in your staff?			H. Human Resources for Health	alpha 240	
q152_pd_doctors	152. What forms of professional development do you provide to your doctors?	152. What forms of professional development do you provide to your doctors ? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.			H. Human Resources for Health	alpha 18	01 = Clinical audits 02 = Surgical audits 03 = Quality assurance meetings 04 = Seminars, conferences, workshops 05 = Support for independent professional development: scholarships 06 = Support for independent professional development: research grants 07 = LGU/DOH led workshops/initiative 08 = No forms of professional development are provided to our doctors 99 = Other (specify)
q152_pd_doctors_other_txt	152. What forms of professional development do you provide to your doctors? — Other (specify) text	152. What forms of professional development do you provide to your doctors? — Other (specify) text			H. Human Resources for Health	alpha 120	
q153_pd_nurses	153. What forms of professional development do you provide to your nurses?	153. What forms of professional development do you provide to your nurses ? READ OPTIONS OUT LOUD. SELECT ALL THAT APPLY.			H. Human Resources for Health	alpha 14	01 = Quality assurance meetings 02 = Seminars, conferences, workshops 03 = Support for independent professional development: scholarships 04 = Support for independent professional development: research grants 05 = LGU/DOH led workshops/initiative 06 = No forms of professional development are provided to our 99 = Other (specify)
q153_pd_nurses_other_txt	153. What forms of professional development do you provide to your nurses? — Other (specify) text	153. What forms of professional development do you provide to your nurses? — Other (specify) text			H. Human Resources for Health	alpha 120	

Validations (CAPI-enforced; HARD = block, SOFT = warn, GATE = display-only)

Item(s)	Rule	Severity
DATE_FIRST_VISITED_THE_FACILITY	Valid date (YYYYMMDD); '20260101 ≤ d ≤ today + 1'	HARD
DATE_OF_FINAL_VISIT_TO_THE_FACILITY	Valid date; '≥ DATE_FIRST_VISITED_THE_FACILITY'; '≤ today + 1'	HARD
TOTAL_NUMBER_OF_VISITS	'≥ 1' when 'ENUM_RESULT_FIRST_VISIT' or 'ENUM_RESULT_FINAL_VISIT = Completed'	HARD

BREAKOFF	Required; defaults to '1 — Continue interview'. If # Continue → terminate; sets 'ENUM_RESULT_FINAL_VISIT' (2 Withdrew → 3 Refused; 3 Postponed → 2 Postponed; 4 Stop-other → 4 Incomplete; 5–7 never-started → 5 Replaced). Enumerator picklists on the Result-of-Visit fields show the paper's four codes only; '5 Replaced' is logic-assigned and swaps in via setvalueset() only on the replacement path (#1290/#1301 class extension, 2026-08-20). Sets 'CASE_DISPOSITION = 2'	HARD
CASE_DISPOSITION	Auto-written by logic, never typed: 0 In progress / 1 Completed / 2 Partial / not completed	—
CLASSIFICATION, REGION, PROVINCE_HUC, CITY_MUNICIPALITY, BARANGAY	Required, non-blank; must exist in the loaded PSGC external lookup dictionaries ('shared/psgc_*.dcf')	HARD
Child PSGC parent consistency	Enforced at pick-time by 'PSGC-Cascade.apc' — 'onfocus' on each child filters its value set to children of the chosen parent, so an inconsistent pair is unrepresentable	HARD — cascade enforces
ENUM_RESULT_FIRST_VISIT = Completed / ENUM_RESULT_FINAL_VISIT = Completed	All Section A–H mandatory items must be non-blank	HARD
FACILITY_GPS_LATITUDE	Alpha; the capture trigger hangs off its 'onfocus'. Protected read-only once 'FACILITY_GPS_READTIME' is non-blank, so coordinates can never be typed	HARD — protect enforces
FACILITY_GPS_LONGITUDE	Alpha; written by the same capture, protected alongside it	HARD — protect enforces
FACILITY_GPS_ALTITUDE	Alpha, from 'gps(altitude)'; no bounds enforced	—
FACILITY_GPS_ACCURACY	Numeric metres. 'ReadGPSReading' warns (msg 1004) when the fix is worse than the 20 m target, but still returns success — the enumerator decides whether to retry	SOFT
FACILITY_GPS_SATELLITES	Numeric, from 'gps(satellites)'	—
FACILITY_GPS_READTIME	Numeric timestamp; doubles as the capture-once sentinel and the protect trigger	—
Failed read	msg 1227 names the budget that elapsed; the radio flag is cleared but the radio is not closed, so a retry resumes the acquisition instead of cold-starting it	SOFT
CAPTURE_VERIFICATION_PHOTO	Gated at preproc on 'ENUM_RESULT_FINAL_VISIT' in {1 Completed, 4 Incomplete} — a Postponed or Refused visit is not photographed, and any stale filename is cleared. Trigger auto-resets to 'notappl' after each attempt	GATE
VERIFICATION_PHOTO_FILENAME	'noinput' — display-only, filled by the camera trigger, never typed. Camera failure warns (msg 1007) rather than trapping the enumerator	SOFT
Filename pattern	'case-{RRPPMMMFFCCC}-verification.jpg', built from 'REGION_CODE', 'PROVINCE_HUC_CODE', 'CITY_MUNICIPALITY_CODE', 'FACILITY_NO', 'CASE_SEQ'	HARD — assigned by the PROC
VERIFICATION_PHOTO_IMAGE	The binary Image item. Photos sync to CSWeb only as binary image dictionary items, off-form — the filename alone does not carry the picture	—
Q3_AGE	'≥ 18'; msg 1056 blocks below the floor	HARD
Q3_AGE	'> 80' warns ("unusually old for an active facility head") without blocking	SOFT
Tenure ≥ 6 months	'Q5_YEARS_AT_FACILITY * 12 + Q5_MONTHS_AT_FACILITY ≥ 6' (IR eligibility). Below it: msg 1075, 'ENUM_RESULT_FINAL_VISIT = 4', 'endlevel' — the interview ends at Q5	HARD — terminates the case
Q5_YEARS_AT_FACILITY	'≤ Q3_AGE - 20' — cannot have run a facility before age 20 (msg 1076)	HARD
Q6_YEARS_HEALTH	'≤ Q3_AGE - 20' (msg 1082)	HARD
Tenure consistency	'Q5_total ≤ Q6_total' — years at this facility cannot exceed total years in any health-related role (msg 1081)	HARD
Q4_SEX	Required, in {1, 2}	HARD
Q2_FACILITY_ROLE	Required; 11 options, no "Other" (drift F1-QC-02 — this spec's older revisions named a 'Q2_DESIGNATION' + '._OTHER_TXT' pair that does not exist)	HARD
Q7_OWNERSHIP	Required; with Q8 it drives the Q139/Q140 PHO gate	HARD
Q8_SERVICE_LEVEL	Required; drives the Q108 value-set swap and the Q117/Q120/Q121 facility-type topics	HARD
Q10_1_UHC_ATTRIB and every other Q<NN>_1_UHC_ATTRIB probe	Reached only when its base is answered Yes; each probe sits on its own screen	GATE
Every .2 detail item (Q12.2, Q13.2, Q14.2, Q15.2, Q16.2, Q17.2, Q18.2, Q19.2, Q35.2)	Reached only on its base's Yes path, immediately after the probe	GATE
Q21_DATA_SUBMIT, Q22_DATA_FREQ, Q23_DATA_REPORTS_USED	The DOH-IS fan; reached only when 'Q20_EMR_USE = Yes'	GATE
Q23_DATA_REPORTS_USED	Check Box, at least one option required (msg 1251)	HARD
Q35_2_PCQM_MEASURES	Check Box, at least one option required (msg 1253)	HARD
Q36_QUALITY_CHALL	Check Box, at least one required (msg 1255); code 09 is exclusive and cannot be combined (msg 1256)	HARD
All Q*_OTHER_TXT	Required, non-blank, when the parent's "Other" code is ticked; cleared and 'noinput' otherwise	HARD
Q39–Q65 entered	'Q38_YK_ACCRED = Yes'	GATE
Q66–Q71 entered	'Q38_YK_ACCRED = No'	GATE
Q72–Q78 entered	'Q38_YK_ACCRED = Yes' (the accredited costing block)	GATE
Q79_MIN_CAP_VALUE_NONACC entered	'Q38_YK_ACCRED = No' only	GATE
Q39_YK_SINCE_YEAR	'2019 ≤ year ≤ current_year' — the UHC Act passed in 2019 (msg 1066)	HARD
Q39_YK_SINCE_MONTH	'1 ≤ m ≤ 12' (msg 1064); and not in the future when the year is the current year (msg 1065)	HARD
Q40_YK_PACKAGE	Check Box, at least one required (msg 1261); code 09 "I don't know" exclusive (msg 1262); code 08 "All of the above" exclusive (msg 1263, #526)	HARD
Q44_CAPITATION_AMT	'≤ 5000' PHP blocks (msg 1071)	HARD
Q44_CAPITATION_AMT	'> 1700' PHP prompts "exceeds the PHP 1,700 PhilHealth max — confirm?" and re-enters on No	SOFT

Q45_PERF_INDICATORS	Check Box, at least one required (msg 1265); code 07 exclusive (msg 1266); code 05 "No requirements" also exclusive (msg 1267, #1188)	HARD
Q47_PAY_FREQ entered	`Q46_KNOW_PAY_FREQ = Yes`	GATE
Q49_TRANCHE_INTERVAL_NUM / Q49_TRANCHE_INTERVAL	Hybrid pair — a Q48 = No skip lands on the `_NUM` half, which is the pair's first field	GATE
Q51_APPLY_REASON	Required, non-blank (msg 1269)	HARD
Q52_ACCRED_DIFFICULT	Check Box, at least one required (msg 1271); code 90 "None of the above" exclusive (msg 1272)	HARD
Q53–Q61 each	Reached only when the matching `Q52_ACCRED_DIFFICULT` option is ticked — aligned 2-character chunk scan, not `pos{}` (#450)	GATE
Q53–Q61 each, when reached	Check Box, at least one option required (msgs 1067, 1320, 1322, 1324, 1326, 1072, ...)	HARD
Q62_ENROLL_RESPONSIBILITY	Check Box, at least one required (msg 1273); code 90 exclusive (msg 1274)	HARD
Q63_ENROLL_INITIATIVES	Check Box, at least one required (msg 1276); code 90 exclusive (msg 1277)	HARD
Q65_ENROLL_CHALL_LIST	Check Box, at least one required (msg 1079)	HARD
Q66_NOT_ACCRED_REASON	Check Box, at least one required (msg 1132)	HARD
Q67 routing	Exactly one of Q68 / Q69 / Q70 / Q71 is answered per case; every branch tail exits to Q79	GATE
Q73_ELIGIBLE_PATIENTS	`> 500,000` warns without blocking (msg 1093)	SOFT
Q74_REGISTERED_PATIENTS	`≤ Q73_ELIGIBLE_PATIENTS` — registered cannot exceed eligible (msg 1094)	HARD
Q78_MIN_CAP_VALUE_ACC	`> 0` and `< 1700` prompts "below the PHP 1,700 PhilHealth max — confirm?" and re-enters on No (#533)	SOFT
Q81_CHARGE_ADDL_CAP_REASONS	Check Box, at least one required (msg 1280)	HARD
Q83_NOT_RECEIVED_REASONS	Check Box, at least one required (msg 1282); code 90 exclusive (msg 1283)	HARD
Q85_PAYMENT_CHALL_LIST	Check Box, at least one required (msg 1285); code 90 exclusive (msg 1286)	HARD
Q86_EXPAND_NEXT	Check Box, at least one required (msg 1288); code 90 exclusive (msg 1289)	HARD
Q87_ADDL_FEATURES	Free text gated on `Q86_EXPAND_NEXT` code 03 being ticked — cleared and `noinput` otherwise, required when it is (msg 1291). The one gated free-text item without an `_OTHER_TXT` suffix	HARD / GATE
Q89–Q94 entered	`Q88_HEARD_BUCAS = Yes`	GATE
Q90_NO_BUCAS_REASON entered	`Q89_HAS_BUCAS = No`	GATE
Q91–Q94 entered	`Q89_HAS_BUCAS = Yes` — the facility actually has a BUCAS Center	GATE
Q91_BUCAS_SERVICES	Check Box, at least one required (msg 1293)	HARD
Q92_BUCAS_FACTORS	Check Box, at least one required (msg 1295)	HARD
Q96–Q98 entered	`Q95_HEARD_GAMOT = Yes`	GATE
Q97_NO_GAMOT_REASON entered	`Q96_GAMOT_ACCRED = No`	GATE
Q98_GAMOT_FACTORS	Check Box, at least one required (msg 1100); entered when `Q96_GAMOT_ACCRED = Yes`	HARD / GATE
Q100–Q104 entered	`Q99_STOCKOUT = Yes`	GATE
Q102_STOCKOUT_AVG entered	`Q101_STOCKOUT_DURATION = 3` (more than 60 days) — below that the month band is already implied, so the item is skipped rather than asked redundantly	GATE
Q103, Q104 entered	`Q95 = Yes` AND `Q96 = Yes` AND `Q99 = Yes` (#384, spec 3.6 gate)	GATE
Q104_ADDR_STOCKOUT_HOW	Required, non-blank (msg 1010); entered only when `Q103_ADDR_STOCKOUT = Yes`	HARD / GATE
Q106–Q121 entered	`Q105_DOH_LICENSED = Yes (1)`	GATE
Q108_DOH_LIC_DIFFICULT value set	Swapped at preproc on `Q8_SERVICE_LEVEL`: `Q108_DOH_LIC_DIFFICULT_PCF_VS1` for a Primary Care Facility, `_HOSP_VS1` for Level 1/2/3 hospitals (#385)	GATE
Q108_DOH_LIC_DIFFICULT	Check Box, at least one required (msg 1228); code 90 "None of the above" exclusive (msg 1229)	HARD
Q109–Q121 each	Reached only when the matching `Q108_DOH_LIC_DIFFICULT` option is ticked; Q109–Q116 map positionally to codes 01–08, Q117–Q121 map to 13, 09, 10, 11, 12 respectively (F1-LOGIC-01)	GATE
Q109–Q121 each, when reached	Check Box, at least one option required (msgs 1298, 1300, 1014, 1302, 1304, 1306, 1308, 1310, 1016, 1312, 1314, 1316, 1018)	HARD
Q117_WHY_DIFF_PRICE_INFO	PCF-only topic — reachable only through Q108's PCF value set	GATE
Q120_WHY_DIFF_EMERG_CART, Q121_WHY_DIFF_ADDONS	Hospital-only topics — reachable only through Q108's hospital value set	GATE
Q123, Q124 entered	`Q122_NBB_CURR = Yes`	GATE
Q124_NBB_BARRIERS	Check Box, at least one required (msg 1110); code 90 exclusive (msg 1230)	HARD
Q126, Q127 entered	`Q125_ZBB_CURR = Yes`	GATE
Q127_ZBB_BARRIERS	Check Box, at least one required (msg 1116); code 90 exclusive (msg 1231)	HARD
Q129_OOP_REASON entered	`Q128_ALLOW_OOP_BASIC = Yes`	GATE
Q131_DIFFICULT_REASON	Required, non-blank (msg 1124)	HARD
Q133_MALASAKIT_WHY entered	`Q132_MALASAKIT_PROVIDED = Yes`; required when reached (msg 1128)	HARD / GATE
Q134_NO_MALASAKIT_WHY entered	`Q132_MALASAKIT_PROVIDED = No`; required when reached (msg 1130)	HARD / GATE
Q136–Q138 entered	`Q135_LGU_SUPPORT = Yes`	GATE
Q136_LGU_SUPPORT_FORMS	Check Box, at least one required (msg 1232)	HARD
Q138_LGU_NOT_SAT_WHY entered	`Q137_LGU_SATISFIED = No`; Check Box, at least one required (msg 1234); code 90 exclusive (msg 1235)	HARD / GATE
Q139, Q140 entered	`Q7_OWNERSHIP = Public` AND `Q8_SERVICE_LEVEL` in Level 1/2/3 Hospital (#386)	GATE
Q140_UNCLEAR_PROTOCOL entered	`Q139_PHO_PROTOCOL_CLARITY` in Unclear / Very Unclear	GATE
Q142_SEND_REFERRAL_HOW	Check Box, at least one required (msg 1237)	HARD

Q143_REFERRAL_FORM_TYPE	Check Box, at least one required (msg 1239); code 05 exclusive (msg 1240)	HARD
Q146_RECEIVE_REFERRAL_HOW	Check Box, at least one required (msg 1028)	HARD
Q147_EXTERNAL_SERVICES_GO	Check Box, at least one required (msg 1030); code 90 exclusive (msg 1242)	HARD
Q149_NOT_SATISFIED_WHY entered	`Q148_REF_SATISFACTION` not in Very Satisfied / Satisfied; required when reached (msg 1032)	HARD / GATE
Q150_HR_CHALL	Check Box, at least one required (msg 1243). The Aug-17 printed list has 5 options and no "I don't know" — ASPSI's #1126 request for one did not survive their own rewrite (clarification item 1)	HARD
Q152_PD_DOCTORS	Check Box, at least one required (msg 1245); code 08 exclusive (msg 1246)	HARD
Q153_PD_NURSES	Check Box, at least one required (msg 1248); code 06 exclusive (msg 1249). Ships without "Clinical audits" / "Surgical audits" per §1 Bug #1	HARD